Fish's Clinical Psychopathology

5th Edition — Comprehensive Study & Viva Notes

Signs & Symptoms in Psychiatry

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A beginner-friendly yet postgraduate-depth learning resource that follows the book's own chapter sequence. Every major phenomenon is explained from first principles, with definitions, simple explanations, clinical examples, differential distinctions, comparison tables, mnemonics, MSE application and four-level viva questions with model answers. All clinical examples are fictional educational illustrations.

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ORIENTATION

How to Use These Notes

These notes follow the **actual chapter order of Fish's Clinical Psychopathology, 5th edition (Casey & Kelly, 2024)**, not a DSM/ICD outline. Each chapter opens with its real page reference from the book, then explains concepts one by one.

READING THE COLOUR CODE

- **Lavender** = formal *Definition* cards.
- **Mint** = *Clinical example* boxes (all fictional illustrations) and key-point summaries.
- **Lemon** = *Mnemonics* and memory aids.
- **Peach** = *Viva traps / common errors / cautions*.
- **Sky** = *Supplementary* material from outside Fish (clearly flagged).
- **Rose expandable boxes** = click to reveal viva questions & model answers (Levels 1–4).

SOURCE & HONESTY NOTE The principal and only Fish source used is the 5th-edition PDF supplied in this repository. There is **no separate "Fish Clinical Cytology"** file — no such book exists in Fish's catalogue; the repository contains exactly one Fish title, *Clinical Psychopathology*, so these notes are built solely from it. Page references are genuine and taken from the book's own printed page numbers. Material added for teaching that is *not* from Fish (e.g., DSM-5-TR terminology updates) is placed in sky-blue "Supplementary" boxes so you always know what is Fish and what is scaffolding. Content has been rephrased in original wording for copyright compliance.

GOLDEN RULE OF DESCRIPTIVE PSYCHOPATHOLOGY A symptom is defined by its **form** (the structure of the experience — e.g., "a perception without an object") not by its **content** (what it is about — e.g., "the voice said "kill yourself""). Most viva marks are lost by describing content and calling it a diagnosis. **Never diagnose from a single symptom or statement.**

CHAPTER 1

Classification of Psychiatric Disorders

Fish's Clinical Psychopathology, 5th ed., Chapter 1 (pp. 1–10)

This opening chapter explains **how** psychiatry groups disorders and **why** the system is still imperfect. Fish begins with a frank admission: a truly definitive classification of any disease must be based on **aetiology** *(the underlying cause)*, but because we rarely know the causes of mental illnesses, psychiatry adopts a **pragmatic, syndrome-based** approach instead.

Syndrome, Symptom and Disease

DEFINITION — SYNDROME A **syndrome** is a constellation of symptoms that is unique *as a group*. Individual symptoms may appear in other syndromes; it is the particular **combination** that makes a syndrome specific.

Simple explanation: a single symptom is one puzzle piece; a syndrome is a recognisable picture formed when several specific pieces regularly appear together. A **disease** is reached when that picture is shown to have one essential cause (often a confirmed physical basis).

WORKED EXAMPLE FROM FISH — SYMPTOM → SYNDROME → DISEASE
Korsakoff's syndrome illustrates the whole progression. First Korsakoff noticed single symptoms (confabulation, impressionability) in people with alcohol dependence. Then a cluster was recognised (disorientation for time/place, euphoria, poor registration, confabulation, "tram-line" thinking). Finally, the discovery of consistent damage to the **mammillary bodies** confirmed it as a true *disease* with a neuropathological basis.

Primary vs Secondary Symptoms

DEFINITIONS **Primary symptom** = the immediate, direct result of the disease process (also used for a symptom not derived from any other psychological event).
Secondary symptom = a psychological elaboration of, or reaction to, a primary symptom.

Example: in mania the elevated mood is primary; the cheerfulness, over-activity, pressure of speech and flight of ideas can all be *understood* as arising from that elevated mood, so they are meaningfully coherent with it. This capacity to empathise with how symptoms arise underlies the primary/secondary distinction.

Early Distinctions: Organic vs Functional

The first major split was between disorders arising from brain disease (**organic**) and those with no obvious such basis (**functional**). Fish stresses these terms are now **literally absurd** — schizophrenia and manic-depression were called "functional", yet genetics and neuropathology show at least some organic basis — but they survive by tradition. DSM-IV renamed "organic mental syndromes" as "delirium, dementia, amnestic and other cognitive disorders."

Organic syndromes: acute, subacute, chronic

Organic syndromes in Fish's framework

Group	Core feature	Examples & notes
Acute	Alteration of **consciousness** (dream-like, depressed or restricted) + disorientation, incoherence, some anterograde amnesia	Four subtypes: *delirium* (dream-like change, hallucinations/ illusions, anxiety, agitation), *subacute delirium* (transitional; lowered awareness, worse at night), *organic stupor/torpor* (poor/no response to stimuli, later amnesia), *twilight state* (restricted consciousness; often well-ordered behaviour, hallucinations common)
With clear consciousness	Consciousness not obviously disordered	*Organic hallucinosis* (e.g., alcohol — auditory hallucinations in clear consciousness, unlike delirium tremens); *amnestic disorders* (e.g., Korsakoff — isolated memory impairment)
Chronic	Dementias (generalised & focal) + amnestic disorders	Generalised: Alzheimer's, Lewy body. Focal: **frontal-lobe dementia** — lack of drive/ foresight, poor planning, indifference to others, sometimes facetious humour (**Witzelsucht**); assessed with

Group	Core feature	Examples & notes
		Wisconsin Card Sorting Test / Stroop Test. Key structures in amnesia: thalamus, hippocampus, mammillary bodies, amygdala.

Functional Syndromes, Neurosis & Psychosis

"Functional" means no readily apparent coarse brain disease. These were traditionally split into **neuroses** and **psychoses**.

Classic (now oversimplified) neurosis vs psychosis distinction

Feature	Neurosis (classic view)	Psychosis (classic view)
Insight	Present	Lacking
Extent of personality affected	Only part	Whole personality distorted
Reality testing	Intact	Builds a false environment from distorted experience

WHY THIS IS AN OVERSIMPLIFICATION (VIVA POINT) Many people with neurotic conditions have *no* insight and deny their illness; some people with schizophrenia willingly seek help. Personality can be badly changed by non-psychotic illness (e.g., depression) yet stay intact in some psychoses (e.g., persistent delusional disorder). Jaspers viewed neurosis as an *abnormal response* transformed by a defence mechanism (e.g., dissociation turning emotion into physical symptoms in conversion disorder). Schneider saw neuroses and personality disorders as quantitative variations of normal — but this breaks down for OCD, whose symptoms differ *qualitatively* from normal.

Terminology drift: "neurotic" and "psychotic" came to mean mild vs severe, or **ego-syntonic** *(compatible with the self-concept, not distressing)* vs **ego-dystonic** *(distressing, incompatible with the self-concept)*. Because of the confusion, DSM-IV removed "neurosis" entirely (continued in DSM-5); ICD-11 replaced the old "neurotic, stress-related and somatoform" group with "anxiety and fear-related disorders", "disorders of bodily distress" and "disorders specifically associated with stress".

Personality Disorders & Psychogenic Reactions

Historically the English-speaking world separated neuroses from personality disorders; the German tradition (Schneider) saw neuroses as reactions of abnormal personalities to mild/moderate stress (or of normal personalities to severe stress). DSM-IV reflected the split using separate axes; the DSM-5 abolished the multiaxial system entirely. **Psychogenic reactions** were reversible, prolonged psychological responses to trauma shaped by personality and culture. The terms "psychogenic" and "neurotic" have been eliminated from recent DSM/ICD. Scandinavian "psychogenic psychoses" survive as **acute and transient psychotic disorders** (ICD-11) / **brief psychotic disorder** (DSM-5).

Modern Classifications: DSM-5 vs ICD-11

DSM vs ICD at a glance

	DSM-5 (APA, 2013)	ICD-11 (WHO, in use since 2022)
Scope	Mental disorders only; mainly US	All medical conditions; one chapter on mental/ behavioural; worldwide
Style	**Operational** criteria — precise, rigorous; good for research/ drug trials; critics call it "Procrustean"	**General descriptions**; allows clinical judgement; more clinically oriented; needs separate research criteria
Big change	Dropped the five axes; expanded diagnoses; removed the bereavement exclusion from major depression (controversial)	Accompanied by Clinical Descriptions & Diagnostic Requirements (CDDR) to curb over-diagnosis

KEY POINT Both DSM-5 and ICD-11 remain **syndrome-based** (built on co-occurring symptoms), *not* on aetiology, psychobiology or prognosis. As DSM-5 taskforce chair Kupfer admitted, *no* laboratory marker specifically identifies any DSM syndrome, comorbidity is very high, and treatment lacks specificity — so the hoped-for aetiology-based classification was never achieved. Publication timeline: DSM-I 1952 DSM-5 2013; ICD-7 1955 ICD-11 2022.

MNEMONIC — THE THREE ACUTE-ORGANIC SUBTYPES BEYOND DELIRIUM
"**D-S-T-T**": **D**elirium, **S**ubacute delirium, organic s**T**upor/torpor, **T**wilight state — all share *disorientation, incoherence, anterograde amnesia*.

L1 **Define a syndrome and distinguish it from a disease.**

A syndrome is a *unique grouping* of symptoms that cluster together; the combination is what makes it specific. A disease is reached when that syndrome is shown to have one essential (usually demonstrable physical) cause. Example: Korsakoff's progressed from symptom syndrome disease once mammillary-body damage was identified.

L2 **Why are the terms "organic" and "functional" considered outdated?**

Because "functional" disorders (schizophrenia, manic-depression) increasingly show genetic and neuropathological bases, so the clean organic/functional dichotomy collapses. Fish calls their literal meaning "absurd" but notes they persist by tradition; DSM-IV renamed "organic mental syndromes" accordingly.

L3 **A researcher wants a homogeneous patient sample for a drug trial. Which classification and why?**

DSM, because its operational, precisely defined criteria yield more homogeneous populations than ICD's descriptive, judgement-based approach. (Note the irony Fish mentions: US Medicare billing is mandated to use ICD codes, not DSM.)

L4 **Both DSM-5 and ICD-11 claim to be "atheoretical". Critically evaluate.**

They are atheoretical about aetiology (deliberately dropping "psychogenic", "neurotic", "functional", "psychosomatic") except for stress-related and substance categories. However, being syndrome-based means they rest on an implicit assumption that co-occurring symptoms map onto real entities — an assumption undermined by absent biomarkers, pervasive comorbidity, diagnostic instability and non-specific treatment response (Kupfer). So "atheoretical" is better read as "aetiology-agnostic by necessity", not a positive scientific achievement.

CROSS-REFERENCES Acute-organic consciousness states Chapter 9. Amnestic/Korsakoff Chapter 5 & Appendix I. Primary vs secondary *delusions* Chapter 4.

CHAPTER 2

What Is Psychopathology? Controversies in Classifying Psychiatric Disorder

Fish's Clinical Psychopathology, 5th ed., Chapter 2 (pp. 11–23)

This chapter is the **conceptual backbone** of the whole book. It answers "what are we actually studying?" and tackles the honest controversies about whether psychiatric disorders are "real".

Jaspers and the Birth of Descriptive Psychopathology

Karl Jaspers — psychiatrist, theologian and philosopher — is called the *father of psychopathology*. His *General Psychopathology* argued that mental illness should be assessed by the **form** of the abnormal phenomena present (hallucination, delusion, thought disorder) rather than their **content**.

> **DEFINITION — FORM VS CONTENT** **Form** = the *structural type* of the experience (e.g., whether a delusion is primary/secondary, systematised/non-systematised).
> **Content** = *what the experience is about* (e.g., persecutory, guilt-laden, nihilistic).

Simple explanation: two patients can both have a delusion (same *form*) but one believes the CIA is watching and the other believes they are damned (different *content*). Jaspers said the diagnostically useful fact is the *presence of a delusion at all*, not its storyline. The psychoanalytic school took the opposite view — that content is a clue to underlying trauma.

Approaches to psychopathology (Fish, Table 2.1)

Descriptive psychopathology	Explanatory psychopathology
Describes phenomena from the person's subjective state; **eschews explanation**	Tries to **explain** phenomena
Works through **form and content**	Cognitive · Behavioural · Psychodynamic/psychoanalytic

KEY POINT Fish's whole book is a text of **descriptive** psychopathology — it carefully describes *what* is experienced and observed, deliberately avoiding premature explanation of *why*.

Two Broad Approaches: Philosophical and Clinical

Philosophical: is mental illness "real"? (after Kendler, 2016)

Three metaphysical stances on psychiatric disorder

Stance	Core claim	Note
Realism	Disorders are real entities (like DNA or a tumour) awaiting discovery of their genes/pathways	The view of most psychiatrists and of biological psychiatry; Kendler cautiously leans here
Constructivism	Disorders have no biological reality; they are social constructs like fashions or music genres	The anti-psychiatry view; some non-antipsychiatry authors worry certain diagnoses arose from the social/political climate (e.g., debates over PTSD, recovered memory)
Pragmatism / utility	Classification is based on what *works* and is useful, not on what is "real" (Jablensky); a middle ground	Kendler calls it "unambitious" and raises ethical concerns; Wakefield fears it weakens the scientific basis of diagnosis

SUPPLEMENTARY — HELPFUL TERMS FROM THIS CHAPTER **Pessimistic induction** (Kuhn): each scientific generation believes it has reached the truth, yet is likely as mistaken as the last — illustrated in psychiatry by discarded "disorders" such as masturbatory insanity, monomania, paraphrenia, hysteria and (the historical declassification of) homosexuality. **Coherence theory of truth**: truth as a set of propositions that "hang together" (Spinoza, Kant, Hegel) — the hoped-for destination as more evidence accumulates.

Clinical: validity and reliability

> **DEFINITIONS** **Validity** = the classification genuinely (a) distinguishes normal from abnormal states (in both non-psychotic and psychotic forms) and (b) distinguishes one disorder from another.
> **Reliability (replicability)** = different clinicians reach the same diagnosis for the same patient.

Both are basic tenets of diagnosis throughout medicine. The problem: psychiatry has **no independent biological markers**, so clinicians depend on the patient's description of symptoms.

The "Clinical Significance" Criterion and False Positives

> **DEFINITION — FALSE POSITIVE DIAGNOSIS** Diagnosing a disorder when none truly exists — e.g., labelling a normal grief response as a depressive episode on symptoms alone. Over-diagnosis inflates prevalence.

DSM-III/IV introduced the requirement that a disturbance cause *"clinically significant distress or impairment"* to reduce false positives — but never defined "clinically significant". Frances warned this hides "the seeds of tautology": symptoms count as a disorder because a doctor judges them to, and the doctor judges them to simply because they came to attention (a vicious circle). Large epidemiological surveys using **lay interviewers** compound this, producing false positives and unstable prevalence figures.

> **SUPPLEMENTARY — WAKEFIELD'S "ZONES OF RARITY" / EXPLANATORY DISCONTINUITY** A valid disorder should show a gap ("zone of rarity") separating cases from non-cases — ideally not only in symptoms but also across aetiology, functioning, course, treatment response and prognosis. Where symptoms fail to delineate, discontinuity in these other domains (**explanatory discontinuity**) may still mark out a valid disorder. Fish notes most DSM-5/ICD-10 disorders do *not* fully meet these requirements.

> **MNEMONIC — THE CHAPTER'S SPINE** "**Jaspers Finds Causes Rarely Valid**" **J**aspers (father) · **F**orm over content · **C**onstructivism/realism/pragmatism · **R**eliability · **V**alidity + clinical significance.

L1 **Who is regarded as the father of psychopathology, and what was his central contribution?**

Karl Jaspers. His central contribution was delineating the *diagnostic features of mental illness by form rather than content*, laying the foundation of descriptive psychopathology (in *General Psychopathology*).

L2 **Distinguish descriptive from explanatory psychopathology.**

Descriptive psychopathology observes and describes the patient's subjective experience through form and content, deliberately avoiding explanation. Explanatory psychopathology (cognitive, behavioural, psychodynamic) attempts to explain *why* phenomena occur. Fish's book is descriptive.

L3 **How does the "clinical significance" criterion try to improve validity, and why is it criticised?**

It requires that symptoms cause significant distress or impairment, aiming to screen out normal reactions and reduce false positives. It is criticised because "clinically significant" was never operationally defined, making it circular (Frances's "seeds of tautology") and unreliable, especially when applied by lay interviewers in epidemiology.

L4 **"Psychiatric disorders are social constructs." Defend and then rebut.**

Defend (constructivism): few disorders have the demonstrable pathology of cancer or Huntington's despite decades of research; some categories appeared in response to social/political climate, and history shows discarded "disorders" (pessimistic induction). *Rebut (realism/multifactorial):* absence of a single marker does not prove unreality — like coronary artery disease, disorders may be defined by a coherent cluster of multifactorial properties (Kendler et al.), moving toward the coherence theory of truth; treating them as mere constructs risks the ethical harm of implying patients are "not really sick".

CROSS-REFERENCES Form/content recurs throughout: delusions (Ch 4), hallucinations (Ch 3). Normal vs abnormal emotion is developed in Chapter 6.

CHAPTER 3 · HIGH-YIELD

Disorders of Perception

Fish's Clinical Psychopathology, 5th ed., Chapter 3 (pp. 24–41)

Perception is how the mind turns sensory stimuli into meaningful experience. Fish divides perceptual abnormalities into two great families:

1. **Sensory distortions** — a *real* stimulus is perceived in a changed way (changed intensity, quality, spatial form, or time).
2. **Sensory deceptions** — the perception does not match the stimulus. These are **illusions** (misinterpretation of a real external object) and **hallucinations** (perception without an adequate external object).

1 · Sensory Distortions

> **DEFINITION** Sensory distortions are changes in perception resulting from a change in the **intensity**, **quality** or **spatial form** of a *real* stimulus.

Changes in intensity — hyper- / hypo-aesthesia

- **Hyperaesthesia** *(increased intensity of sensation)* from intense emotion or a lowered physiological threshold — e.g., roof tiles seen as flaming red; a door closing heard like thunder. **Hyperacusis** *(heightened sensitivity to noise)* occurs in anxiety, depression, alcohol hangover and migraine. Very bright, intense colours occur in hypomania, epileptic aura, LSD use, and even in intense normal states (religious fervour, being in love).
- **Hypoacusis** / hypoaesthesia *(reduced sensation)* in delirium (raised threshold + attentional defect speak louder and slower to delirious patients), depression and attention-deficit disorder. In depression everything may look black or all food taste the same.

Changes in quality

Mainly visual, often drug-induced: **xanthopsia** (yellow), **chloropsia** (green), **erythropsia** (red). The metallic taste of lithium is a *true change in gustation*, not a hallucination. In derealisation everything looks unreal and strange; in mania objects can look perfect and beautiful.

Changes in spatial form — dysmegalopsia

Spatial-form distortions

Term	Meaning
Micropsia	

Term	Meaning
	Objects seen smaller than they really are (includes objects seeming to retreat into the distance without size change — some call that *porropsia*)
Macropsia / megalopsia	Objects seen larger than they are
Metamorphopsia	Objects seen as irregular/distorted in shape
Dysmegalopsia	Umbrella term for change in perceived size/shape (some restrict it to one side larger than the other)

Causes: retinal disease, disorders of accommodation/convergence, but most commonly **temporal and parietal lobe lesions**; rarely schizophrenia.

Distortions of the experience of time

Personal (not physical) time is affected by illness. In severe/psychotic depression time drags or seems to stand still; in mania it races. Some patients with schizophrenia feel time moves in fits and starts (sometimes with a delusional elaboration that clocks are interfered with) and show **age disorientation**. Acute organic states cause temporal disorientation.

2 · Sensory Deceptions — Illusions

> **DEFINITION — ILLUSION** A **illusion** is a **misinterpretation of a real external stimulus**: stimuli from a perceived object are combined with a mental image to produce a false perception.

Simple explanation: something *is* really there, but you read it wrongly — the shadow on a dark road becomes an attacker. Illusions are **not in themselves pathological**; they occur in healthy people, especially with inattention, strong emotion, or a raised perceptual threshold (as in delirium).

Three types of illusion (after Sims)

Type	Mechanism	Example
Completion illusion	Inattention; the mind completes a gap from experience	Reading faded "–ook" as "book"; missing a misprint

Type	Mechanism	Example
Affect illusion	Arises from a particular mood state	A bereaved person momentarily "sees" the deceased; a frightened delirious patient perceives innocent gestures as threats
Pareidolia	Vivid illusions from excessive fantasy/ imagery, *without effort* and sometimes against the will	Seeing vivid pictures in fire or clouds

VIVA TRAP — PAREIDOLIA IS SPECIAL Pareidolia differs from ordinary illusions because it needs **no effort and no particular mood/mind-set** — it is driven by vivid imagery. So it cannot be explained by affect or set, which is exactly why examiners ask about it.

3 · Hallucinations

DEFINITIONS — HALLUCINATION Classic: *"a perception without an object."* Because that does not cover functional hallucinations (see below) and must exclude dreams, **Jaspers** defined it as *"a false perception which is not a sensory distortion or a misinterpretation, but which occurs at the same time as real perceptions."* SCAN simply calls them "false perceptions".

Simple explanation: the experience feels like a true perception coming from *outside* ("without"), but it actually comes from *within*. This "from within but experienced as from without" quality is what separates a hallucination from a vivid **mental image** (which also comes from within but is recognised as such).

Essential features (true perception vs mental image — Jaspers)

Jaspers' criteria distinguishing true perceptions from mental images

True perception	Mental image
Substantial; appears in **objective (outer) space**	Incomplete; exists in **subjective (inner) space**
Clearly delineated, constant	

True perception	Mental image
	Not clearly delineated; inconstant, must be re-created
Independent of the will	**Dependent on the will**
Sensory elements full and fresh	Sensory elements sketchy

Pseudo-hallucinations — the classic controversy

DEFINITION — PSEUDO-HALLUCINATION (JASPERS) A type of **mental image** that, although clear and vivid, lacks the substantiality of a true perception. It is experienced in **full consciousness**, is **known not to be a real perception**, is located in **subjective (inner) space** (e.g., "inside the head"), yet, like true hallucinations, is **involuntary**.

TWO COMPETING DEFINITIONS (EXAMINERS LOVE THIS) **Jaspers**: defined by *inner vs outer space* — pseudo-hallucinations sit in inner space and are variants of imagery, so lack the diagnostic weight of true hallucinations. **Hare**: defined by *degree of insight* — but insight fluctuates on a continuum, which (Fish notes) makes the whole concept of pseudo-hallucination largely redundant. Jaspers said there is no gradual transition between true and pseudo-hallucinations; **Fish disagreed**, arguing for a continuum (supported by Leff's sensory-deprivation work). SCAN avoids the term entirely but rates insight and inside/outside-the-head location separately.

CLINICAL SIGNIFICANCE The usual teaching is that pseudo-hallucinations do *not* necessarily indicate serious illness, whereas true hallucinations do — but this only holds if you fix a definition. If insight is the criterion and it fluctuates, the distinction loses meaning.

Causes of hallucinations

Intense emotion · psychiatric disorder · suggestion · disorders of a peripheral sense organ · sensory deprivation · CNS disorders.

- **Emotion:** very depressed patients with guilt hear *fragmentary, disjointed* reproachful voices ("rotter", "kill yourself") — unlike the continuous voices of schizophrenia or organic hallucinosis. Continuous persistent voices in "depression" should raise suspicion of schizophrenia or physical disease.
- **Suggestion:** normal people can be made to hallucinate (dim-corridor experiments, hypnosis).
- **Peripheral sense-organ disease: Charles Bonnet syndrome** — complex visual hallucinations with reduced vision, *no psychopathology*, clear consciousness, common in the elderly; important as a differential.
- **Sensory deprivation:** normal subjects hallucinate after hours of minimal stimulation; may contribute to post-cataract-surgery delirium and paranoid states in the deaf.

- **CNS:** diencephalic/cortical lesions; hypnagogic & hypnopompic hallucinations are special organic types.

Hallucinations by modality

Hallucinations in each sense

Modality	Key points from Fish
Auditory (once called "phonemes")	Elementary (noises/bells) → partly organised (music) → fully organised (voices). "Voices" are characteristic of schizophrenia: 2nd/3rd-person, running commentary, imperative ("imperative hallucinations"). **Gedankenlautwerden** = hearing one's own thoughts spoken aloud just before/as they occur; **écho de la pensée** = hearing them *after* ("thought echo"). Thought broadcasting and thought echo are best classed as disorders of *thought*, not perception.
Visual	Elementary (flashes) → organised (people, animals, scenic "cinema" hallucinations). Common in **acute organic states with clouding**; small animals/insects typical of delirium tremens (with fear). **Uncommon in schizophrenia** — their presence should cast doubt on that diagnosis. **Lilliputian hallucinations** = tiny people/objects, usually with pleasure/amusement.
Olfactory (phantosmia)	Schizophrenia, organic states, rarely depressive psychosis; temporal-lobe aura (e.g., burning rubber). Must be separated from *olfactory reference syndrome* (belief one emits a smell, often inferred from others' behaviour). Pleasant religious smells = "Padre Pio phenomenon".
Gustatory (phantageusia)	Schizophrenia and acute organic states; hard to separate from delusional explanation. Often co-occurs with olfactory hallucinations.
Tactile / superficial	**Formication** = feeling small animals crawling on the skin; the "cocaine bug" in cocaine psychosis (with

Modality	Key points from Fish
	persecutory delusions). Sims' subtypes of superficial: *thermic, haptic, hygric, paraesthetic*.
Kinaesthetic	Muscles/joints feel twisted, pulled, moved; schizophrenia (distinguished from passivity by the *definite sensation*); vestibular variants (sinking, flying) in organic states.
Visceral (deep)	Twisting/tearing pains; bizarre ("organs torn out"). **Delusional zoopathy** = belief an animal is in the body, with a hallucinatory component (the patient *feels* it).

Special kinds of hallucination

Type	Definition & example
Functional hallucination	A stimulus triggers the hallucination, and **both the stimulus and the hallucination are perceived together** as separate things. E.g., voices heard *only when* a tap runs — but the running water is also heard. (Contrast with illusion, where the stimulus is *transformed*.)
Reflex hallucination	A stimulus in **one** sense modality produces a hallucination in **another**. A morbid form of **synaesthesia**. E.g., feeling a pain in the head (somatic) whenever others sneeze (auditory stimulus).
Extracampine hallucination	A hallucination **outside the sensory field** — e.g., seeing someone behind you while looking ahead, or hearing voices talking "in London" while you are in Liverpool. Can occur in health (hypnagogic), schizophrenia, epilepsy.
Autoscopy (phantom mirror-image)	Seeing oneself *and knowing it is oneself* — requires kinaesthetic/somatic sensation too, so not purely visual. Linked to parieto-occipital lesions, epilepsy; the "Doppelgänger" folklore.

Type	Definition & example
Hypnagogic / hypnopompic	Occur when *falling asleep* (hypnagogic) / *waking* (hypnopompic). True hallucinations but **not pathological**; also seen in narcolepsy (hypnopompic a better narcolepsy indicator).

Hallucinatory syndromes (hallucinosis)

Persistent hallucinations in any modality *without* other psychotic features. **Alcoholic hallucinosis**: usually auditory, in relative abstinence, clear sensorium, rarely >1 week. **Organic hallucinosis**: in 20–30% of dementias (esp. Alzheimer), auditory/visual, with disorientation and memory impairment.

The sense of "presence"

Hard to classify — neither a true sensory deception nor a delusion. The vivid feeling that someone is present (often behind/beside one) though nothing is seen or heard. Occurs in health, organic states, schizophrenia and dissociation.

Body image distortions

Term	Meaning
Hyperschemazia	Perceived magnification of body parts (painful/ paralysed limbs feel larger; anorexia nervosa's "fatness" is the best-known)
Hypo-/a-schemazia	Body parts perceived as diminished/absent (parietal lesions); must be separated from nihilistic delusion
Paraschemazia	Distortion/twisting/splitting of body parts (hallucinogens, epileptic aura, migraine)
Hemisomatognosia	Acting as if one side of the body is missing (migraine, epileptic aura)
Anosognosia	"Denial of illness" — e.g., denying left-sided hemiplegia after right-hemisphere stroke
Somatoparaphrenia	Delusional beliefs about a limb (e.g., it belongs to someone else)

SUPPLEMENTARY **Koro** (penis retracting, fatal) — South-East Asia, a faulty anatomical belief; closest modern equivalent is an anxiety disorder. **Gerstmann syndrome** (dominant parietal): agraphia, acalculia, finger agnosia, right/left disorientation.

MNEMONICS **Illusion vs hallucination:** "*ILLusion = ILL-reading of something real; haLLucination = no object at aLL.*"
Three illusion types: "**CAP**" = **C**ompletion, **A**ffect, **P**areidolia.
Functional vs reflex hallucination: *Functional = same modality, stimulus still perceived; Reflex = different modality (one sense triggers another).*"

FICTIONAL CLINICAL EXAMPLE — DISTINGUISHING ILLUSION, FUNCTIONAL & REFLEX HALLUCINATION

Illustrative only. A patient reports: "When the extractor fan runs, I hear it **and** I hear a voice saying my name — two separate sounds." **Functional hallucination** (same modality, stimulus still heard, hallucination gone when fan off).
Contrast: "At dusk I thought the coat on the door was a man." **Illusion** (real stimulus transformed).
Contrast: "Every time I hear a door bang, I feel an electric shock in my spine." **Reflex hallucination** (auditory stimulus somatic hallucination, different modality).

Clarifying question: "When you hear the voice, is the fan noise still there at the same time, or does the fan noise *turn into* the voice?" (Still there + separate = functional; turns into = illusion.)

Eliciting symptoms (Fish's suggested probes)

PROBE QUESTIONS (ADAPT TO EACH PATIENT)

"Do you ever hear voices when there is nobody about and nothing to explain them?" · "Do you hear people talking when there is nobody there?" · Follow-up: "Do you know if other people hear these voices too?"
Visual: "Do you ever have visual experiences, such as seeing things that are not there?"

Viva Questions — Perception

L1 **Define hallucination, illusion and pseudo-hallucination.**

Hallucination — a perception without an adequate external object, arising from within but experienced as coming from without, occurring alongside real perceptions (Jaspers). **Illusion** — a misinterpretation of a real external stimulus. **Pseudo-hallucination** — a vivid but non-substantial mental image, in inner (subjective) space, recognised as not real, experienced in full consciousness, but involuntary (Jaspers).

L2 **How do you distinguish a functional hallucination from an illusion?**

Both arise in response to an environmental stimulus. In a **functional hallucination** the stimulus and the hallucination are perceived *simultaneously and separately* — the stimulus is not changed (e.g., the tap still sounds like a tap while a voice is also heard). In an **illusion** the stimulus itself is *transformed* and becomes an integral part of the new false perception.

L2 **Why is the modality of a hallucination diagnostically important?**

Auditory hallucinations in clear consciousness suggest schizophrenia (or affective psychosis); fragmentary reproachful voices suggest depression. Visual hallucinations suggest an *organic/ acute confusional state* and are uncommon in schizophrenia — their presence should prompt a search for organic cause or Charles Bonnet syndrome. Tactile formication suggests cocaine or alcohol states. So modality steers the differential.

L3 **A patient denies hearing voices but says "everybody here is talking about me in Greek." How do you proceed?**

Explore whether this is (a) a *delusion without a hallucination* ("people talk about me" with no actual auditory experience), or (b) continuous hallucinatory voices *attributed to real people* (a delusional elaboration of a hallucination). Ask: "Do you actually *hear* them speaking, or do you just *know* they are talking about you?" Fish's Greek-patient vignette shows how careful content-analysis revealed continuous hallucinated Greek voices driving "motiveless" attacks — crucial for risk assessment.

L4 **Critically discuss the clinical utility of the concept of pseudo-hallucination.**

Its utility depends entirely on the definition chosen. On Jaspers' *spatial* definition (inner vs outer space) it marks imagery-like experiences of lesser diagnostic weight. On Hare's *insight* definition it collapses, because insight varies on a continuum, rendering the category — and its supposed reassurance value — redundant. Fish sided with a *continuum* between pseudo- and true hallucinations (unlike Jaspers), and SCAN sidesteps the term, rating insight and location separately. Practical conclusion: record the *actual phenomenology* (location, insight, substantiality) rather than forcing a label.

CROSS-REFERENCES Thought echo / broadcasting classed as thought disorder Ch 4. Hallucinations in clouded consciousness (delirium) Ch 9. Body-image distortions overlap self-disorders Ch 7. Charles Bonnet, Cotard, Capgras Appendix I.

CHAPTER 4 · HIGH-YIELD

Disorders of Thought and Speech

Fish's Clinical Psychopathology, 5th ed., Chapter 4 (pp. 42–62)

Fish groups the disorders of thinking into: disorders of **intelligence**; disorders of the **stream** of thought; disorders of the **possession** of thought (obsessions, compulsions, thought alienation); disorders of the **content** of thought (delusions, overvalued ideas); and disorders of the **form** of thought (formal thought disorder). Speech disorders are treated at the end.

MASTER DISTINCTION FOR THE WHOLE CHAPTER **Thought form** = *how* thoughts are structured and connected (reasoning). **Thought content** = *what* is believed (belief). "Disorders of reasoning vs disorders of belief." Keep these apart in every answer.

Disorders of Intelligence

DEFINITION **Intelligence** = the ability to think and act rationally and logically; measured by problem-solving and concept-formation using words, numbers, symbols and patterns. IQ tests are built to a mean of 100, SD 15.

Learning (intellectual) disability bands: **borderline 70–90, mild 50–69, moderate 35–49, severe 20–34, profound <20**. **Dementia** = loss of intelligence from brain disease, with disturbance of multiple cortical functions. The cognitive deficits of schizophrenia ("schizophrenic dementia" historically) are *not* a true dementia — they are part of the psychopathology.

Three legitimate uses of "to think"

- **Undirected fantasy ("autistic"/dereistic) thinking** — daydreaming, not goal-bound.
- **Imaginative thinking** — does not exceed the rational/possible.
- **Rational/conceptual thinking** — aims to solve a problem.

Disorders of the Stream of Thought

Subdivided into **tempo** (speed) and **continuity** (flow).

Tempo — Flight of ideas

DEFINITION **Flight of ideas** — thoughts follow each other rapidly with **no general direction**; successive thoughts are linked by *chance* connections that can usually still be

understood (clang/verbal associations, assonance, alliteration, puns, proverbs). Speech is easily diverted by external stimuli and superficial internal associations.

Simple explanation: like dominoes — each thought shares just one "half" with the next, so you can follow each hop but there is no destination. **Typical of mania.**

FISH'S OWN EXAMPLE (MANIA) *Illustrative.* Asked where she lived, a manic patient replied: *"Birmingham, Kingstanding; see the king he's standing, king, king, sing, sing, bird on the wing, wing, wing on the bird, bird, turd, turd."* — note the clang/rhyming chain.

Prolixity ("ordered flight of ideas") = a milder hypomanic form: many irrelevances and lively embellishment, but the patient can still return to the goal. Flight of ideas *with* pressure of speech is usual; without pressure of speech it is claimed in some mixed states.

Tempo — Inhibition / slowing of thinking

Train of thought slowed; fewer ideas/images; experienced as difficulty deciding, poor concentration, loss of clarity, often a strange "fuzziness in the head". Seen in depression and (rarely) manic stupor. May be mistaken for dementia.

Tempo — Circumstantiality

DEFINITION **Circumstantiality** — thinking proceeds slowly with many unnecessary, trivial details, but the goal is **eventually reached**. The point is never lost; it is reached by a convoluted path.

Associated historically with epilepsy-related personality traits, also learning disability, obsessional traits, schizophrenia, schizoaffective disorder.

Continuity — Perseveration

DEFINITION **Perseveration** — a mental operation (word, phrase or idea) **persists beyond the point at which it is relevant**, blocking further progress.

Example: asked the previous PM he says "David Cameron"; asked the *current* PM he again says "David Cameron. No, I mean David Cameron." Worse with harder tasks. **Strongly suggests organic brain disease.** Differs from *verbal stereotypy* (same word regardless of situation, not a volitional problem).

Continuity — Thought blocking

DEFINITION **Thought blocking** — a **sudden arrest** of the train of thought, leaving a "blank"; an entirely new thought may then begin.

In patients retaining insight it can be terrifying. When clearly present it is **highly suggestive of schizophrenia** — but exhaustion/anxiety can mimic losing one's thread.

Obsessions, Compulsions & Disorders of Possession of Thought

DEFINITION — OBSESSION (RUMINATION) A thought that **persists and dominates** despite the person recognising it as purposeless or beyond the point of usefulness. It appears **against the will**, is recognised as the person's *own*, is resisted, and is typically repugnant (so the prudish are tormented by sexual thoughts, the religious by blasphemy, the timid by violent images).

Compulsions = obsessional *motor acts* (e.g., washing driven by a contamination obsession). Because an obsession must be resistible and recognised as one's own, there are obsessional *images, ideas, fears, impulses* — but **no** obsessional hallucinations or moods. Contemporary shift: earlier writers stressed sexual obsessions; now fears of *doing harm* predominate (e.g., a mother fearing she will harm her baby).

Thought alienation

DEFINITION **Thought alienation** — the experience that one's thoughts are **under external control** or that others participate in one's thinking. Unlike obsessions, the thoughts are experienced as *foreign* and outside one's control.

Forms of thought alienation (now in ICD-11 schizophrenia criteria)

Form	Experience
Thought insertion	Thoughts are **put into** the mind from outside and recognised as foreign
Thought withdrawal / deprivation	Thoughts suddenly **disappear**, withdrawn by a foreign influence (?subjective experience of thought blocking / "omission")
Thought broadcasting / diffusion	As one thinks, everyone else thinks in unison / thoughts escape and are accessible to others (several competing definitions — record verbatim)

Ego-syntonic vs ego-dystonic: as a person with psychosis gains insight, thought alienation tends to feel increasingly ego-dystonic and distressing.

VIVA TRAP — OBSESSION VS THOUGHT INSERTION Obsession: thought is recognised as **my own**, intrudes against my will, I resist it. Thought insertion: thought is

experienced as **not mine**, placed by an external agent. Possession/ownership is the deciding feature.

Disorders of the Content of Thought — Delusions

DEFINITION — DELUSION A **false, unshakeable belief that is out of keeping with the person's social and cultural background**. Falseness is *not* its essential quality (a jealous delusion may be true by coincidence); the essence is that it is held with unusual conviction, is unmodifiable by evidence, and is not culturally sanctioned.

Delusion vs overvalued idea vs delusion-like idea

	True (primary) delusion	Secondary / delusion-like idea	Overvalued idea
Origin	Primary — cannot be derived from any other morbid phenomenon	Understandably derived from another morbid phenomenon (mood, hallucination)	Understandable, with some basis in reality
Onset	Often abrupt	Follows the primary phenomenon	Gradual
Conviction / plausibility	Implausible content, held with relative indifference to others' opinions	Variable	Less fixed than a delusion; plausible

DEFINITION — OVERVALUED IDEA A thought that, because of its strong **feeling tone**, takes precedence over all other ideas and **keeps that precedence for a long time**. Less fixed than a delusion and with some basis in reality.

Primary delusions (apophany) — Schneider's three forms

Form	Description
Delusional mood (Wahnstimmung)	A pervasive sense that "something is going on that concerns me", but the patient doesn't yet know what. Usually resolves into a delusional idea/perception.
Delusional perception	A **new, self-referential meaning** attached to a *normally perceived* object, not derivable from mood/attitude. Schneider's "two-memberedness": link 1 = perceiving the object; link 2 = the new significance. *E.g.* "the spoon had a crown I am of royal descent."
Sudden delusional idea (autochthonous delusion)	A delusion appears **fully formed**, out of the blue ("brain-wave"). No two-memberedness — the delusion is contained within a single experience/ memory.

VIVA TRAP — DELUSIONAL PERCEPTION VS DELUSIONAL MISINTERPRETATION A patient with persecutory delusions hears the stairs creak and "knows" it is a detective. That is **delusional misinterpretation** (derived understandably from the existing persecutory belief/mood), *not* a true delusional perception — because in a delusional perception the new meaning *cannot* be understood from the prevailing affect or attitudes.

Secondary delusions & systematisation

Arise understandably from other morbid experience (depressed mood, hallucinations, stress, personality). "Sensitive" personalities may develop an understandable paranoid psychosis (*sensitiver Beziehungswahn*, now a delusional disorder). In schizophrenia primary experiences get elaborated into a system ("delusional work"); delusions may be **systematised** (one basic delusion, logically built up) or non-systematised.

Content of delusions

- **Persecution** — including delusions of *reference* (people talking about/spying on me), poisoning/ infection (often "explanatory"), and *influence* (passivity "explained" by rays, hypnotism, the internet). *Note:* "paranoid" strictly means "delusional", not "persecutory".
- **Infidelity** (morbid jealousy / Othello) — organic & functional; strongly linked to alcohol dependence; dangerous (violence, homicide).
- **Love / erotomania** (de Clérambault / "fantasy lover syndrome").

- **Grandiose** — variable, from "I am God" to inflated talents; historically GPI (neurosyphilis), now mania.
- **Ill health (hypochondriacal)** — characteristic of depression; may involve family; overlaps body dysmorphic disorder.
- **Guilt** — in severe depression; may become grandiose ("the most evil person ever").
- **Nihilistic (delusions of negation)** — denying existence of body/mind/world; may be linked to delusions of enormity (e.g., refusing to urinate to avoid "flooding the world"). Cotard syndrome.
- **Poverty** — typical of depression (now less common).

THE REALITY OF DELUSIONS Not all patients act on delusions; in chronic illness there is often a gap between belief and behaviour (the patient "who is God" happily stays as a voluntary inpatient). But guilt/hypochondriacal delusions (without retardation) and infidelity delusions carry real suicide/homicide risk.

Disorders of the Form of Thought (Formal Thought Disorder)

DEFINITION **Formal thought disorder (FTD)** = disorder of conceptual/abstract thinking, most common in schizophrenia and organic brain disorders — a disturbance of *how* thoughts connect, not *what* they contain.

Classic descriptions of FTD

Author	Key concepts
Bleuler	Loosening of associations; *condensation* (two ideas blended into one false concept), *displacement* (one idea used for an associated one), misuse of symbols *concrete thinking*
Cameron	*Asyndesis* (lack of adequate connections), *metonyms* (imprecise substitute expressions), **over-inclusion** (can't exclude irrelevant material)
Goldstein	Loss of the abstract attitude concrete thinking (vocabulary preserved, unlike organic disorders)
Schneider (5 features)	*Derailment, substitution, omission, fusion, drivelling*

Schneider's three healthy-thinking features — *constancy, organisation, continuity* — map to three objective thought disorders:

- **Transitory thinking** — derailments, substitutions, omissions; grammar/syntax disturbed. (Omission ≠ desultory: in omission the *intention* is interrupted leaving a gap.)
- **Drivelling thinking** — all the constituents of a complex thought are present but muddled together; organisation lost.
- **Desultory thinking** — grammatically correct speech, but simple intrusive ideas force their way in.

Speech Disorders

Term	Definition
Stammering/ stuttering	Flow interrupted by pauses/repetition of word fragments; may reappear at onset of acute schizophrenia or in severe anxiety
Mutism	Complete loss of speech (hysteria, depression, schizophrenia, organic). Elective/selective mutism in children. Akinetic mutism (upper mid-brain/ diencephalon lesions)
Talking past the point (Vorbeireden)	Replies show the question was understood but the answer is of a related, wrong topic ("colour of grass?" "White"). Seen in Ganser (approximate answers), hebephrenic schizophrenia, catatonia
Neologism	A genuinely new word, or an ordinary word used in a new way (schizophrenia). Distinguish from *paraphasia* (aphasia) and *malapropism* (misused real word, no psychiatric significance)
Schizophasia ("word salad", speech confusion)	Profoundly confused speech despite possibly preserved non-verbal work; regarded as a form of thought disorder

VIVA TRAP — THOUGHT DISORDER VS DYSPHASIA/APHASIA Schizophasia/ neologisms stem from disordered *thought* (vocabulary and grammar machinery intact). Aphasic paraphasias/word-dumbness stem from *language* apparatus damage. Over-inclusion

and concrete thinking with preserved vocabulary point to FTD; loss of vocabulary points to organic language disorder.

MNEMONICS **Schneider's 5 FTD features:** "**DSO-FD**" = **D**erailment, **S**ubstitution, **O**mission, **F**usion, **D**rivelling.
Circumstantial vs tangential: "*Circumstantial Comes back; Tangential Trails off.*"
Three primary delusional experiences: "Mood Perception Idea" (delusional **M**ood, delusional **P**erception, sudden delusional **I**dea).

Viva Questions — Thought & Speech

L1 **Define a delusion. Is falseness its essential feature?**

A false, unshakeable belief out of keeping with the person's social/cultural background. **Falseness is not the essential feature** — a jealous delusion may happen to be true; the essence is the abnormal certainty, unmodifiability and cultural incongruity, together with its arising independently of other morbid phenomena (if primary).

L2 **Differentiate flight of ideas from loosening of associations.**

In **flight of ideas** the links between consecutive thoughts *can still be understood* (chance/clang/verbal associations), speed is increased, and it is typical of mania. In **loosening of associations / derailment** the links are *not understandable* — thought slides onto unrelated subsidiary tracks — and it is typical of schizophrenia and reflects formal thought disorder.

L2 **Obsession vs delusion vs overvalued idea — key separators?**

Obsession: recognised as one's own, resisted, ego-dystonic, insight retained. **Delusion:** held with full conviction, unshakeable, not resisted, insight absent, culturally incongruent. **Overvalued idea:** strong feeling tone, dominant and long-lasting, plausible, with some basis in reality, less fixed than a delusion. The patient's attitude (resistance, conviction, ownership) is the discriminator.

L3 **At interview, how would you demonstrate a delusional perception rather than a delusional misinterpretation?**

Elicit the perceived object and the meaning attached, then test whether the new meaning is *understandable* from the existing mood/beliefs. If the patient already feels persecuted and now "reads" a creaking stair as a spy, that is derivable misinterpretation. If a completely neutral, normally perceived object suddenly acquires a self-referential meaning that cannot be derived from affect or prior attitude (Schneider's two-memberedness), that is a true delusional perception.

L4 **Thought broadcasting has several definitions. Why does this matter, and how should you document it?**

Fish/Hamilton defined it as "everyone thinks in unison with me"; others define it as thoughts escaping/being accessible, sometimes requiring prior audibility (Gedankenlautwerden). Because the term covers slightly different experiences with different implications, you should **record the patient's belief verbatim** rather than just the label, so colleagues know whether an auditory component is present and can map it onto ICD-11 criteria accurately.

CROSS-REFERENCES Thought echo/broadcasting also discussed as perception Ch 3. Passivity of *action/feeling* (vs thought) Ch 7 & Ch 8. Retrospective & delusional memories Ch 5. Specific delusional syndromes (Capgras, Cotard, Othello, de Clérambault) Appendix I.

CHAPTER 5

Disorders of Memory

Fish's Clinical Psychopathology, 5th ed., Chapter 5 (pp. 63–70)

Memory types: **sensory** (fades in seconds) **short-term / working** **long-term**. The process of remembering has four parts: **registration, retention, retrieval, recall**. The hippocampus stores explicit (declarative) memory and transfers short- to long-term; implicit (procedural) memory lives in the limbic system, amygdala and cerebellum.

Memory subtypes

Type	Meaning
Explicit / declarative	Conscious remembering. *Episodic* (specific events, incl. autobiographical) + *semantic* (abstract facts)
Implicit / procedural / skills	Prior learning expressed without active awareness (typing, swimming)
Flashbulb memory	Vivid autobiographical memory of an emotionally arousing event — recalled confidently but *not necessarily accurately*

Memory disorders divide into **amnesias** (loss) and **paramnesias** (distortion). Normal failure also occurs via decay and interference: *proactive* (old interferes with new learning) vs *retroactive* (new interferes with old recall).

The Amnesias

> **DEFINITION** **Amnesia** — partial or total inability to recall past experiences/events; organic or psychogenic.

Amnesia	Key features
Dissociative (hysterical) amnesia	Sudden, during extreme trauma; loss of *personal identity* and personal events, but complex behaviours preserved; may have a fugue. Hard to separate from malingering in forensic cases

Amnesia	Key features
Katathymic amnesia (motivated forgetting)	Inability to recall *specific* painful memories via repression; **no loss of personal identity**; more circumscribed/persistent; underpins "recovered memory" (disputed as "false memory")
Retrograde amnesia	Loss of memory for events *before* an injury (usually brief)
Anterograde amnesia	Loss for events *after* the injury (failure to encode)
Blackouts	Circumscribed anterograde amnesia in alcohol dependence; indicate reversible brain damage
Amnestic (amnesic) state	Subacute coarse brain disease: can't register new info; remote memory intact. Korsakoff (thiamine deficiency) is the classic. Not diagnosed if dementia or clouded consciousness present
Depressive pseudodementia	Amnesia from impaired concentration in depression; resolves with treatment

Ribot's law: in progressive chronic brain disease, **recent memories are lost before remote ones**.

Paramnesias (Distortions of Memory)

Distortions of recall

Term	Definition
Retrospective falsification	Unintentional distortion as memory is filtered through current mood (e.g., depressed person recalls only failures)
False memory	Confident recollection of an event that never happened (not mere distortion). Related: *memory distrust syndrome*, *source amnesia*

Term	Definition
Screen memory	A partly true, partly false recollection — only part is recalled because the whole truth is too painful
Confabulation	Falsification of memory in **clear consciousness with organic pathology**; gaps filled with untrue/imagined material, not corrected. Two types: *embarrassed* (common; filling an evident gap) and *fantastic* (wild, excess detail)
Pseudologia fantastica	Fluent plausible/pathological lying *without* organic pathology (antisocial/hysterical personality); admits lying when confronted with proof. Munchausen's is a variant
Cryptamnesia	"Not remembering that one is remembering" — e.g., quoting another's passage believing it original
Retrospective / delusional memory	Backdating delusions ("I was always persecuted"); a primary delusional experience taking the form of a memory

VIVA TRAP — CONFABULATION VS PSEUDOLOGIA FANTASTICA VS LYING
Confabulation = organic, unintentional, fills memory gaps. **Pseudologia fantastica** = non-organic, fluent, often at a time of crisis, admitted when confronted. Both differ from ordinary lying (conscious, for clear gain). Ganser's *approximate answers* and malingering are further differentials.

Distortions of recognition

- **Déjà vu** — feeling a current event has been experienced before (and *déjà entendu*, *déjà pensé*). **Jamais vu** — a genuinely familiar event feels unfamiliar. Seen in health and temporal-lobe epilepsy.
- **False recognition (reconnaissance)** — positive (strangers taken for friends) or negative (friends taken for disguised strangers). **Capgras syndrome** = belief that a familiar person has been replaced by a double.

Hyperamnesia = exaggerated registration/retention/recall. **Flashbacks** (sudden intrusive, emotionally charged memories of trauma — core to PTSD) differ from mere intrusive recollections (which lack the emotional familiarity). **Eidetic images** = near-hallucinatory vivid visual memories.

MNEMONIC"**R-R-R-R**" memory process: **R**egistration, **R**etention, **R**etrieval, **R**ecall. Ribot = **R**ecent lost before **R**emote.

L1 **Define anterograde and retrograde amnesia.**

Retrograde = loss of memory for events *before* the injury/illness onset; anterograde = inability to form/recall new memories for events *after* it (failure of encoding).

L3 **How would you distinguish confabulation from pseudologia fantastica at interview?**

Look for organic cognitive impairment and clear consciousness (confabulation) vs an intact sensorium with a dramatic crisis context (pseudologia). Confabulation fills genuine memory gaps and is not corrected; pseudologia is fluent, grandiose, and the person concedes untruth when confronted with incontrovertible evidence. Corroborate with cognitive testing and collateral history.

L4 **Comment on the forensic significance of false memory and dissociative amnesia.**

Memory is reconstructive, not a camera; retrospective falsification, source amnesia and suggestibility can generate confident false memories (even false confessions via memory-distrust syndrome). Dissociative amnesia for offences is hard to separate from malingering. Hence the need for corroboration, caution with "recovered memories" (the false-memory debate), and awareness that statutes of limitation partly reflect memory's fallibility.

CROSS-REFERENCES Korsakoff/amnestic syndrome also in Ch 1 & Appendix I. Delusional memory Ch 4. Repression as a defence Appendix II.

CHAPTER 6

Disorders of Emotion

Fish's Clinical Psychopathology, 5th ed., Chapter 6 (pp. 71–79)

CORE VOCABULARY **Feeling** = subjective positive/negative reaction to an experience. **Emotion** = a stirred-up state with physiological change. **Mood** = a *pervasive and sustained* emotion that colours one's view of the world (describe intensity, duration, fluctuation, type). **Affect** = short-lived present emotional responsiveness, *inferred* by the clinician from body language/face; described as within normal range, constricted, blunt or flat — and may be congruent or incongruent with mood.

Fish organises emotion into **five clinical levels**: (1) normal emotional reactions, (2) abnormal emotional reactions, (3) abnormal expressions of emotion, (4) morbid expressions of emotion, (5) morbid disorders of emotion.

1 · Normal emotional reactions

Understandable, culturally appropriate responses (grief; distress at a serious diagnosis). Normal grief usually settles over ~6–12 months, with only brief functional incapacity.

2 · Abnormal emotional reactions

Understandable but with prolonged functional impairment (the **Yerkes–Dodson curve**: performance improves then deteriorates past a stress threshold). Diagnostically = **adjustment disorders**.

- **Anxiety** — "fear for no adequate reason"; may have physical symptoms; sudden combined physical symptoms = **panic attack**.
- **Anxious foreboding** — a sense something terrible will happen without knowing what.
- **Phobia** — fear restricted to one object/situation/idea, with avoidance.
- **Reactivity of mood** — mood fluctuates with the environment.
- **Demoralisation** — isolation, despair, feeling trapped; source seen as *external*, **anhedonia and guilt absent, motivation intact** (unlike depression). Studied in cancer.

3 · Abnormal expressions of emotion (patient is aware of them)

Term	Meaning
Dissociation of affect	Lack of emotional response where some is expected (numbness) — unconscious defence against

Term	Meaning
	overwhelming stress; underlies the "unresponsive" feeling in depersonalisation/derealisation
La belle indifférence	Serene lack of concern about gross conversion symptoms
Denial	Unaware refusal to acknowledge a clear event (distinguish from the conscious avoidance it is often wrongly used for)
Apathy	Emotional indifference / lack of motivation (schizophrenia, cannabis amotivational syndrome, deprivation) — not the same as dissociation of affect
Perplexity	Bewildered, puzzled state (anxiety, mild clouding, emerging schizophrenia)

4 · Morbid expressions of emotion (patient *un*aware of the morbidity)

- **Blunting / inadequacy of affect** ("parathymia", Bleuler) — loss of emotional life, social inappropriateness (schizophrenia).
- **Incongruity of affect** — emotion mismatched to context (laughing at bad news).
- **Flattening / constriction** — reduced range, but what is shown is in the appropriate direction (unlike incongruity).
- **Lability of affect** — rapid abrupt mood changes unrelated to stimuli (mixed states, mania, borderline PD, frontal/cerebrovascular damage).
- **Affective incontinence** — *total* loss of control (cerebral atherosclerosis, MS); "forced laughing/weeping" with a mismatch between expression and subjective feeling.
- The **"smiling depressive"** — retains the communicatory smile but the eyes stay tense/unchanged; a dangerous pitfall in suicide-risk assessment.

5 · Morbid disorders of emotion

Self-sustaining pathological states (don't resolve when a stressor is removed). **Depressive illness** is the commonest: morbid sadness ("a weight/cloud/darkness"), psychomotor retardation, **anhedonia** *(loss of the capacity for interest/pleasure; term coined by Ribot, 1896)*, loss of *emotional resonance* (feeling of unreality, inability to feel), diurnal variation, early-morning wakening; may reach depressive stupor. **Morbid anxiety** can present as agitation. **Morbid euphoria/elation** in mania/hypomania (unrelated to events, not modified by sad influences; pressure of speech, flight of ideas,

disinhibition). **Ecstasy** = exalted rapturous state (religious experience, post-partum) — distinct from mere elation. Frontal euphoria with silliness/lack of foresight = **moria / Witzelsucht**.

VIVA TRAP — MANIA VS HYPOMANIA (FISH) The distinction lies in the presence of **psychotic symptoms** (typically grandiose delusions) and/or "marked" functional impairment in mania — though DSM-5 does not define "marked".

MNEMONIC — FIVE LEVELS OF EMOTION "**N**ormal **A**lways **A**voids **M**orbid **M**adness" **N**ormal reactions **A**bnormal reactions **A**bnormal expressions **M**orbid expressions **M**orbid disorders.

L1 **Distinguish mood from affect.**

Mood is a pervasive, sustained emotion that colours the person's whole outlook (the "climate"); affect is the immediate, observable, short-lived emotional responsiveness inferred from the patient's face and body (the "weather"). Affect may be congruent or incongruent with mood.

L2 **Differentiate blunting, flattening and incongruity of affect.**

Blunting/inadequacy = loss of emotional finesse/social sensitivity. **Flattening/constriction** = reduced *range*, but responses still point the right way. **Incongruity** = emotion in the *wrong direction* for the context (laughing at tragedy). All are "morbid expressions" because the patient is unaware of them.

L3 **How does demoralisation differ from a depressive episode, and why does it matter?**

Demoralisation: external source, feeling trapped/uncertain about direction, but *anhedonia and guilt absent and motivation preserved*. Depression: pervasive low mood, anhedonia, guilt, loss of emotional resonance, often diurnal/biological features. It matters because demoralisation (common in cancer) may not respond to antidepressants and calls for meaning-centred/ psychological support rather than automatic pharmacotherapy.

L4 **Why is the "smiling depressive" clinically dangerous?**

The retained communicatory smile (lips but not eyes) can beguile the clinician into underestimating depth of depression and suicide risk. Fish advises probing the most sensitive areas and watching for depression to break through; expressions of empathy may unmask it. This is a documentation and risk-assessment hazard.

CROSS-REFERENCES Depersonalisation/derealisation & loss of emotional resonance Ch 7. Psychomotor retardation / stupor Ch 8. Depressive delusions Ch 4.

CHAPTER 7

Disorders of the Experience of Self

Fish's Clinical Psychopathology, 5th ed., Chapter 7 (pp. 80–85)

Jaspers described **four aspects of self-experience** (Ichbewusstsein / "self-experience"):

1. Awareness of the **existence and activity** of the self.
2. Awareness of being a **unity** at any moment in time.
3. Awareness of **continuity of identity** over time.
4. Awareness of being **separate from the environment** (ego boundaries).

Disturbance of awareness of self-activity

DEFINITION — DEPERSONALISATION The patient feels they are **no longer their normal, natural self**, often with a feeling of unreality. It is always, to some degree, **unpleasant** (which separates it from ecstatic states).

DEFINITION — DEREALISATION The associated feeling that the **environment** is flat, dull, distant and unreal.

Simple explanation: depersonalisation = "*I* feel unreal / like a robot / cut off from myself"; derealisation = "the *world around me* feels unreal / like a stage set / behind glass." Both occur with retained insight and clear consciousness (ICD-11/DSM-5 depersonalisation–derealisation disorder); roughly twice as common in women; reported at least once by 30–70% of young adults.

VIVA TRAP — DEPERSONALISATION VS NIHILISTIC DELUSION
Depersonalisation is **not a delusion** and insight is retained ("I *feel as if* I don't exist"). A nihilistic delusion is a mood-congruent *belief* held with conviction ("I *am* dead / I have no brain", as in Cotard). Severe depression can occasionally present with prominent depersonalisation and be mistaken for schizophrenia because the patient struggles to describe it.

Loss of emotional resonance — in depression the normal flow of feeling toward people/objects is lost ("I cannot feel"), often most painful toward loved ones, intensifying guilt.

Disturbance of immediate awareness of self-unity

Feeling "as if" one is two people (depersonalisation/depression). People with appreciation-needing personalities or learning disability may drop the "as if". Patients with certain delusions (e.g., demoniac possession) or schizophrenia may feel they are two people.

Disturbance of continuity of self

Feeling of not being the same person as before ("I died under my old name and came to life as a new person"; "born again"). Rarely, **dissociative identity disorder** (multiple personality) — strongly linked to trauma/childhood abuse; differentials include other dissociative disorders, schizophrenia, bipolar, borderline PD, malingering, complex partial epilepsy.

Disturbance of the boundaries of the self (ego boundaries)

Many schizophrenic symptoms represent a breakdown of the self/world boundary:

- **Passivity phenomena ("made"/"fabricated" experiences)** — actions, impulses or feelings experienced as **made by an external force** ("I am a guided missile"). The patient *knows* the act is not their own.
- **Thought alienation** (insertion/withdrawal/broadcasting) — the thinking equivalent (see Ch 4).
- Belief that one's actions/thoughts have excessive effect on the world ("when I urinate, bad things happen to others").

VIVA TRAP — PASSIVITY VS OBSESSION VS "AS-IF" ANXIETY In **obsession/compulsion**, control over the act is lost but *ownership is retained* (it is still "my" impulse, resisted). In **passivity**, both control *and* ownership are lost — the act is "made" by an outside agency. In severe **anxiety**, a patient may say "as if" they are controlled; the preserved "as if" and intact insight separate this from true passivity.

Theory of mind, consciousness & schizophrenia

Theory of mind / mentalisation = ability to infer others' mental states. Deficits are linked to autism and to paranoid symptoms (not specific to, nor invariable in, schizophrenia). Sass & Parnas argue schizophrenia is fundamentally a **self-disorder** (increased self-consciousness, diminished self-affection).

MNEMONIC — JASPERS' 4 ASPECTS OF SELF "**A-U-C-B**" = **A**ctivity/existence, **U**nity, **C**ontinuity, **B**oundaries.

ELICITING (FISH'S PROBES)

"Are there times when you feel you are no longer your normal self, or a feeling of unreality about yourself or the world?" · "Do you sometimes feel as if you are living in a dream, or the world is less than real?" · "Are your thoughts, emotions and actions under your control, or does it ever feel as if some are controlled from outside?"

L1 **Define depersonalisation and derealisation.**

Depersonalisation — the unpleasant experience of feeling unreal, detached from or no longer one's natural self, with insight retained. Derealisation — the associated feeling that the external world is unreal, flat and distant.

L2 **How do passivity phenomena differ from obsessions?**

In passivity, ownership of the thought/action/feeling is lost — it is experienced as made by an external agent. In obsession, ownership is retained (it is recognised as one's own intrusive thought) even though control is impaired and it is resisted.

L3 **A depressed patient says "I don't exist any more." How do you decide if this is depersonalisation or a nihilistic delusion?**

Probe the quality of conviction and insight. Depersonalisation: an "as-if" quality with retained insight ("I *feel* unreal, though I know I'm here"). Nihilistic delusion: a held belief with full conviction, unshakeable, mood-congruent ("I *am* dead / have no insides"). Clarify with gentle reality-testing and look at the surrounding depressive picture.

L4 **Evaluate the claim that schizophrenia is fundamentally a disorder of the self.**

Sass & Parnas propose a unified "ipseity disturbance" account — hyper-reflexivity (increased self-consciousness) and diminished self-affection — that can generate passivity, thought alienation and boundary loss. It elegantly links disparate first-rank-type symptoms to a single self-boundary disturbance and connects to theory-of-mind research. Limitations: theory-of-mind deficits are neither specific to nor universal in schizophrenia, and the model remains descriptive/phenomenological rather than established at a mechanistic level.

CROSS-REFERENCES Thought alienation Ch 4. Subjective passivity of motor acts Ch 8. Loss of emotional resonance / dissociation of affect Ch 6. Cotard, Couvade Appendix I.

CHAPTER 8

Motor Disorders

Fish's Clinical Psychopathology, 5th ed., Chapter 8 (pp. 86–102)

Mostly objective motor signs, but Fish opens with a **subjective** one.

Subjective motor disorder — alienation of motor acts

In **obsessions/compulsions**, ownership of the act is retained though it appears against the will. In **schizophrenia**, the act may be experienced as foreign/made by an outside influence — **ideas or delusions of passivity**, often "explained" by rays, hypnosis, the internet. Severe anxiety can produce "as-if" versions — distinguishing these from true passivity is crucial to avoid misdiagnosis.

Classification of objective motor disorders

Adaptive movements · non-adaptive movements · motor speech disturbances · disorders of posture · abnormal complex behaviour · medication-related movement disorders.

Disorders of adaptive movements

- **Expressive movements:** reduced in depression (incl. "smiling depression" mask); stiff masked face in catatonia; "ointment face" in post-encephalitic Parkinsonism; **Schnauzkrampf** (snout spasm). Exaggerated in mania.
- **Reactive movements:** automatic adjustments to new stimuli; lost/obstructed in catatonia and stupor; prompt/excessive in anxiety.
- **Goal-directed movements:** **psychomotor retardation** (depression — like a steady brake) vs **obstruction/blocking (Sperrung)** (catatonia — like a rod in the spokes: irregular hindrance; the patient may start then stop halfway, or respond only at the last moment). **Mannerism** = an odd, stilted but still *goal-directed* movement/posture.

> **VIVA TRAP — MANNERISM VS STEREOTYPY** **Mannerism** = an unusual, stilted variation of a *goal-directed* movement (the goal is still visible). **Stereotypy** = a repetitive, *non-goal-directed* movement/posture carried out uniformly. Mannerisms are not diagnostic of any disorder.

Disorders of non-adaptive movements

Term	Definition
Tic	

Term	Definition
	Sudden involuntary twitch of small muscle groups (often face); may be psychogenic or organic (post-encephalitic, Huntington's, Tourette's)
Stereotypy	Repetitive, uniform, non-goal-directed movement or utterance (remnant of a goal-directed act may be discernible)
Perseveration (motor)	Senseless repetition of a goal-directed act after its purpose is served (induced movement). *Palilalia* (word repeated with rising frequency) & *logoclonia* (last syllable repeated) are special forms
Verbigeration	Continuous repetition of sentences/fragmented word-strings, monotonous tone

Abnormal induced movements (elicited by the examiner)

The catatonic compliance/resistance spectrum

Sign	What happens
Automatic obedience	Carries out every instruction regardless of consequence (Kraepelin's tongue-and-pin test)
Echopraxia	Imitates the examiner's actions
Echolalia	Echoes words/phrases, even in unknown languages (*echologia* = echoes the content in different words)
Forced grasping / grasp reflex	Grasps the examiner's hand despite instructions not to; grasp reflex = grasps any object placed in the hand (frontal lobe sign). *Magnet reaction* = hand follows the withdrawing finger
Mitmachen (co-operation)	Body can be moved into any position despite being told to resist; returns to rest when released (moderate effort needed)

Sign	What happens
Mitgehen	Extreme co-operation — body moves with the *slightest* pressure ("anglepoise lamp" sign), returns to rest
Gegenhalten (opposition)	Opposes passive movement with *equal* force to that applied
Negativism	Motiveless resistance to all interference; passive (resists) or active/command (does the opposite)
Ambitendency	A series of tentative to-and-fro movements that never reach the goal (ambivalence in action)

Disorders of posture

- **Waxy flexibility (flexibilitas cerea)** — plastic "soft-wax" resistance as the limb is moved; the imposed posture is then held.
- **Catalepsy / perseveration of posture** — imposed/fortuitous postures maintained for long periods (1 min, often longer), *without* the waxy resistance.
- "Psychological pillow" — head held inches off the pillow for hours.

Abnormal complex patterns of behaviour — Stupor and Excitement

> **DEFINITION — STUPOR** A state of near-complete loss of activity with no reaction to external stimuli; an extreme form of hypokinesia. Occurs in shock, dissociative/conversion disorders, depression, psychosis, catatonia and organic disease.

Catatonic vs depressive stupor

Feature	Catatonic stupor	Depressive stupor
Face	Deadpan, expressionless (eyes often lively)	Depressed/anxious facies
Muscle tone	Usually increased/ variable; obstruction	Normal
Catalepsy, stereotypies	Present	Absent

Feature	Catatonic stupor	Depressive stupor
Response to emotional stimuli	Absent	Present (worsens on affect-laden topics)
Incontinence	Common	Absent

Excitement often occurs in the same illnesses as stupor (manic, catatonic, organic, psychogenic). **Pathological drunkenness (mania à potu)** = senseless violence after a *small* amount of alcohol, no ataxia, with amnesia. **Extended suicide** = a depressed, deluded person kills loved ones ("better off dead") before dying by suicide. **Dissociative fugue** = dissociative amnesia + apparently purposeful journey.

Medication-related movement disorders

Akathisia (restlessness, with subjective tension), **acute dystonia** (sustained spasm), **tardive dystonia**, acute & **tardive dyskinesia** (repetitive purposeless orofacial movements). Note dyskinesias occur in schizophrenia *before* any medication, so examine and rate at baseline.

MNEMONICS **Compliance spectrum (increasing):** Gegenhalten (opposes) Mitmachen (co-operates, moderate effort) Mitgehen (moves with slightest touch). "*Gegen fights, Mit goes along, Mitgehen goes **all** the way.*"
Retardation vs obstruction: "*Retardation = steady Brake; Obstruction = Rod in the spokes.*"

L1 **Define waxy flexibility.**

A form of catatonic posturing in which the examiner feels a smooth, plastic ("soft-wax") resistance while moving the patient's limb, after which the imposed, often uncomfortable, posture is maintained.

L2 **Differentiate mannerism from stereotypy.**

A mannerism is an odd, stilted exaggeration of a *goal-directed* act (the goal is still apparent). A stereotypy is a uniform, repetitive, *non-goal-directed* movement or utterance. Neither is diagnostic of a specific illness on its own.

L3 **At the bedside, how would you distinguish catatonic from depressive stupor?**

Examine facial expression (deadpan vs depressed), muscle tone (increased/variable + obstruction vs normal), presence of catalepsy/stereotypies/waxy flexibility (catatonic) and incontinence (catatonic). Test emotional reactivity: the depressive stuporose patient becomes more distressed on affect-laden topics, whereas the catatonic patient typically shows no emotional response. Always exclude organic/neurological causes (EEG, imaging, LP) in rapidly developing stupor.

L4 **Why must movement disorders be examined before starting antipsychotics?**

Because spontaneous dyskinesias and neurological soft signs occur in drug-naïve first-episode schizophrenia; a baseline examination (and validated rating scales) separates illness-related from drug-induced movements, informs later causality judgements about tardive dyskinesia, and supports prevention/management — important given evidence that such movement disorders affect quality of life and may be mortality risk factors.

CROSS-REFERENCES Subjective passivity Ch 7. Psychomotor retardation / stupor also in depression Ch 6. Stupor as an organic state Ch 9. Dissociative fugue Ch 9 & Appendix I.

CHAPTER 9

Disorders of Consciousness

Fish's Clinical Psychopathology, 5th ed., Chapter 9 (pp. 103–107)

DEFINITION **Consciousness** (for descriptive psychopathology) = a state of **awareness of the self and the environment**. Its intensity varies even in the fully awake person.

Attention first

Active attention (deliberate focus) vs **passive attention** (stimuli grab attention effortlessly) — reciprocally related. Disturbed active attention = **distractibility** (fatigue, anxiety, depression, mania, schizophrenia, organic states).

Orientation and "confusion"

Orientation is tested in **time** **place** **person** — and consciousness disturbance tends to erode them in that order (person lost last). Disorientation in a recent-onset illness is a *prima facie* sign of an acute organic state. "Confusion" strictly means disoriented for time and place (though it is loosely used for "muddled/bewildered/perplexed", which also occur in severe anxiety and acute schizophrenia *without* disorientation).

KEY POINT Consciousness can change in **three basic ways**: it may be *dream-like*, *depressed (lowered)*, or *restricted*.

1 · Dream-like change of consciousness (delirium)

Lowered consciousness (raised threshold for stimuli); disoriented for time and place but *not* person; **visual hallucinations** prominent (small animals, fear/terror); cannot separate mental images from perceptions; disordered dream-like thinking (displacement, condensation, misuse of symbols). Lilliputian hallucinations (with pleasure) may occur. **Occupational delirium** = carrying out the actions of one's trade. Worse at night; "toxic confusional state" is a milder form. ICD-11 standardises delirium terminology (rapid onset, fluctuating course).

2 · Lowering of consciousness (torpor)

Psychologically benumbed, slowed, apathetic, may perseverate, **without** hallucinations/illusions/delusions/restlessness. Historically from severe infection; now often post-stroke. May be mistaken for severe dementia but shows remarkable partial recovery.

3 · Restriction of consciousness (twilight state)

DEFINITION — TWILIGHT STATE A condition with **restriction** of morbidly changed consciousness, a **break in the continuity** of consciousness, and relatively **well-ordered behaviour**; sudden onset and end, variable duration, sometimes unexpected violent/emotional behaviour. The commonest cause is epilepsy.

Hysterical twilight state = restriction from unconscious motives; the person may "forget" personal identity as a temporary escape. **Fugues** (wandering + amnesia) are not all hysterical — a depressed person may wander before attempting suicide. ICD-11 places dissociative fugue under dissociative disorders.

MNEMONIC — THREE WAYS CONSCIOUSNESS CHANGES "**D-L-R**" = **D**ream-like (delirium) · **L**owered (torpor) · **R**estricted (twilight). Orientation lost in order "**T-P-P**" = Time Place Person.

ELICITING (FISH'S PROBES)

"Are there times you feel especially distractible, as if you cannot focus?" · "Do you find it hard to remember what time/day it is, or where you are, or who the people around you are?" · "Do you sometimes feel as if you are living in a dream, or drifting without focus?"

L1 **In what order is orientation lost as consciousness is impaired?**

Time first, then place, then person (person is retained longest because people supply identifying information).

L2 **Name and distinguish the three basic changes of consciousness.**

Dream-like (delirium): raised threshold, prominent visual hallucinations, fear, image/perception confusion. **Lowered** (torpor): benumbed, slowed, apathetic, *no* hallucinations/restlessness. **Restricted** (twilight state): consciousness narrowed to a few ideas, broken continuity, relatively ordered behaviour, often epileptic.

L3 **Why can delirium be mistaken for dementia, and how do you avoid the error?**

Both show poor grasp and cognitive failure, but delirium has acute onset, fluctuating (often night-worse) course, clouded/dream-like consciousness and perceptual disturbance, whereas dementia is chronic with clear consciousness. Check onset and course, test orientation and attention, look for fluctuation and visual hallucinations, and investigate for organic causes (bloods, imaging, EEG). Torpor and institutionalised chronic schizophrenia are further mimics.

L4 **Discuss the difficulty of attributing a twilight/fugue state to "hysterical" motives.**

Twilight states can be organic (classically epileptic) or psychogenic; fugues may be dissociative, depressive (pre-suicidal wandering) or malingered. It is often impossible to be certain how much motivation is unconscious vs deliberate, and prior head injury/concussion may shape a dissociative presentation. Hence cautious formulation, exclusion of epilepsy/organic causes, collateral history and avoidance of prematurely labelling behaviour "hysterical".

CROSS-REFERENCES Acute organic states Ch 1. Visual hallucinations in clouded consciousness Ch 3. Stupor/akinetic mutism Ch 8. Dissociative fugue Appendix I.

CHAPTER 10

Personality Disorders

Fish's Clinical Psychopathology, 5th ed., Chapter 10 (pp. 108–121)

DEFINITIONS **Personality** = the totality of a person's emotional and behavioural traits characterising day-to-day living.
Personality disorder = deeply ingrained, maladaptive patterns of behaviour, usually recognisable by adolescence and continuing through adult life.

History: Hippocrates' four humours four temperaments (choleric, melancholic, sanguine, phlegmatic). **Schneider** (greatest modern influence) defined the person with personality disorder as one who *"suffers or makes society suffer on account of their abnormality."*

Dimensions vs categories

Trait psychology places traits on a continuum from absent severe; only traits crossing a **severity threshold** (harming self or others) are pathological. Most studies find 4–5 dimensions (e.g., Costa & McCrae's five-factor model: neuroticism, extraversion, openness, agreeableness, conscientiousness). ICD-11 abandons categories for a **dimensional severity model**.

Assessing personality — the golden rule

CRITICAL ASSESSMENT PRINCIPLE Assess personality **when the person has recovered from the current episode of illness**, because mental-state disorder contaminates the picture (a depressed person may describe lifelong worthlessness; hypomanic irritability may look like irascibility). If assessing during illness, rely on **informants** who know the premorbid, lifelong traits. A trait is only a disorder if it **negatively impacts the person and/or others**.

Instruments mentioned: MMPI, Eysenck Personality Inventory (affected by current illness via the N scale); screeners (IPDS, SAPAS, PAS-Q); questionnaires (MCMI-IV, PDQ-4, NEO-PR — risk of over-diagnosis/false positives); structured interviews (PAS, DIPD-IV, SIDP, Personality Disorder Examination, SCID-5-PD). Reliability is higher with informants than with subjects alone.

DSM-5 clusters

DSM-5 three clusters

Cluster	Label	Disorders
A	"Eccentric" (odd)	Paranoid · Schizoid · Schizotypal
B	"Dramatic"	Antisocial · Borderline · Histrionic · Narcissistic
C	"Fearful" (anxious)	Avoidant · Dependent · Obsessive–compulsive

Note: ICD-11 classifies **schizotypal with schizophrenia** (not personality) and **omits narcissistic PD**.

Category sketches (DSM-5) with key differentials

Disorder	Core picture	Key differential (from Fish)
Paranoid	Touchy, mistrustful, suspects malevolence, litigious, pathologically jealous; may decompensate into psychosis	Persistent delusional disorder; Diogenes syndrome in old age
Schizoid	Aloof, detached, emotionally cold, little interest in relationships, prefers solitary/abstract pursuits	Avoidant PD (wants relationships but can't), prodromal schizophrenia, ASD, depersonalisation (schizoid's detachment is *not* distressing)
Schizotypal	Aloof but with some involvement; odd communication, inappropriate affect, ideas of reference, magical thinking	Prodromal schizophrenia (classed with schizophrenia in ICD-11)
Histrionic	Self-dramatisation, labile mood, sexual	Depression/hypomania (short-lived histrionic

Disorder	Core picture	Key differential (from Fish)
	provocativeness, egocentricity, craves praise; shallow/ manipulative	features); controversial, charge of sexism
Borderline (EUPD)	Instability of affect, behaviour, mood, self-image; impulsivity, self-harm, emptiness, fear of abandonment, splitting; micropsychotic episodes	Antisocial PD (BPD lacks the callous core), bipolar; trauma history common
Antisocial (dissocial)	Callousness, lack of empathy/remorse, thrill-seeking, little learning from punishment; conduct disorder in childhood	Substance-misuse-driven criminality; **most violent people do NOT have this disorder**
Obsessive–compulsive (anankastic)	Punctuality, neatness, need for control, rigidity, risk-avoidance; often *over*-diagnosed as traits are seen as virtues	Don't diagnose merely for liking routine — require distress/negative impact
Dependent	Excessive reliance on others, lacks confidence/ ambition, compliant, fears being alone	Post-bereavement/ depressive dependence (resolves)
Avoidant (anxious)	Acute need for friendship but shy, tense, low self-worth, fears criticism	Social anxiety disorder (more circumscribed), schizoid PD (no desire for relationships)
Narcissistic	Grandiose self-importance, fantasies of success, needs	

Disorder	Core picture	Key differential (from Fish)
	admiration, exploitative, fragile self-esteem	Rarely diagnosed outside the US; omitted from ICD-11

VIVA TRAP — SCHIZOID VS AVOIDANT VS DEPERSONALISATION **Schizoid:** no desire for relationships, detachment is *not* distressing. **Avoidant:** intense desire for relationships but held back by shyness/low self-worth. **Depersonalisation:** a *distressing* "cut-off/outside oneself" feeling, state-based, not a lifelong trait.

ICD-11 — the "monumental" change

ICD-11 replaces categories with a **dimensional model based on severity** (personality *difficulty*, then *mild / moderate / severe* personality disorder), plus up to **six trait domains** and one **borderline qualifier**:

ICD-11 trait domains + borderline qualifier

Domain	Essence
Negative affectivity	Broad, intense negative emotions out of proportion; lability, low self-esteem, mistrust
Detachment	Social and emotional withdrawal; aloofness, limited emotional expression
Dissociality	Self-centredness + lack of empathy; deceptive, manipulative, sometimes aggressive/callous
Disinhibition	Rash action on immediate stimuli; impulsivity, distractibility, irresponsibility, recklessness
Anankastia	Rigid perfectionism and control; orderliness, inflexibility, risk-avoidance
Borderline qualifier (not a trait domain)	Instability of relationships/self-image/affect + impulsivity; requires 5 of 9 features; flags need for psychotherapy

ICD-11 also adds **secondary personality change** (due to a medical condition) and reclassifies "enduring personality change after catastrophe" under **complex PTSD**.

MNEMONICS **Clusters:** "A = Weird, B = Wild, C = Worried" (A eccentric/odd, B dramatic, C anxious).
ICD-11 six domains: "**Negative Detached Dissocials Disinhibit Anankastic-ally**" Negative affectivity, Detachment, Dissociality, Disinhibition, Anankastia (+ Borderline qualifier).

L1 Define personality disorder.

A deeply ingrained, maladaptive pattern of behaviour, generally recognisable by adolescence and persisting through adult life, that causes the person to suffer and/or makes others suffer (Schneider).

L2 Why should personality be assessed after recovery from illness?

Because a current mental-state disorder contaminates self-report and presentation — depression can mimic lifelong low self-worth, hypomania can mimic irascibility. Lifelong premorbid traits, ideally confirmed by informants, are what define personality; this reduces false-positive personality-disorder diagnoses.

L3 How would you separate avoidant PD from social anxiety disorder?

Avoidant PD is more pervasive (fear across many social situations) with pervasive low self-esteem and an excessive need for acceptance as part of a lifelong trait pattern; social anxiety disorder is more circumscribed (e.g., public speaking/eating in front of others) without the global low self-worth. Fish notes considerable overlap, so some view avoidant PD as a mild/severe end of social anxiety.

L4 Compare the ICD-11 and DSM-5 approaches to personality disorder and critique them.

DSM-5 retains the 10 categorical diagnoses in 3 clusters despite poor inter-rater reliability (except antisocial) and heavy overlap/comorbidity. ICD-11 abandons categories for a severity-based dimensional model with five trait domains plus a borderline qualifier — more empirically grounded but clinically less familiar and harder to navigate. The anomalous retention of the borderline qualifier (critiqued by Tyrer) and omission of narcissistic PD show the compromises involved. The dimensional move better captures the trait continuum and comorbidity but at some cost to clinical communicability.

CROSS-REFERENCES Paranoid PD vs persistent delusional disorder Ch 4. Depersonalisation vs schizoid detachment Ch 7. Splitting/projective identification as defences Appendix II. Diogenes, Othello, Munchausen Appendix I.

APPENDIX I

Psychiatric Syndromes (Eponymous & Named)

Fish's Clinical Psychopathology, 5th ed., Appendix I (pp. 122–126)

These named syndromes are classic viva material. All definitions follow Fish's own descriptions.

Named psychiatric syndromes

Syndrome	Core feature / underlying psychopathology
Blocq's disease (astasia-abasia)	Inability to walk/stand normally; bizarre gait with near-falls and last-moment recovery — a conversion (dissociative motor) disorder
Briquet's syndrome	Somatisation disorder: multiple physical complaints across systems, no physical cause, onset <30, chronic
Capgras syndrome	Belief a close person has been replaced by an exact double — *delusional misidentification*, not hallucination
Fregoli syndrome	Belief that different people met are really one persecutor in disguise
Syndrome of **intermetamorphosis**	Belief people have swapped identities (total psychological + physical transformation, not disguise)
Syndrome of **subjective doubles**	Delusion that the patient has a double/ doppelgänger
Reduplicative paramnesia	Delusional belief that identical places/events exist

Syndrome	Core feature / underlying psychopathology
Charles Bonnet syndrome	Visual hallucinations with visual impairment, *no* other psychopathology, clear consciousness
Cotard syndrome	Nihilistic delusion of being dead/rotting/non-existent; SCAN treats it as psychotic depersonalisation/derealisation
Couvade syndrome	A man develops pregnancy-like symptoms during his partner's pregnancy — not delusional; akin to conversion/empathy
De Clérambault's syndrome (erotomania)	Delusion that an exalted, usually inaccessible person is in love with the patient
Othello syndrome	Morbid jealousy — delusion/overvalued idea of partner's infidelity; dangerous (stalking, violence)
Diogenes syndrome	Gross self-neglect, squalor, hoarding (syllogomania) in the (often elderly, intelligent) reclusive; ~50% no psychiatric illness
Ekbom's syndrome	Restless legs syndrome — unpleasant leg sensations at rest, worse at night, urge to move
Ganser syndrome	Approximate answers (Vorbeireden) + clouding of consciousness + hallucinations + amnesia; prisoners/stress; "hysterical pseudodementia"
Korsakoff's syndrome	Amnestic disorder from thiamine deficiency in chronic alcohol misuse; recent-memory impairment, apathy, confabulation

Syndrome	Core feature / underlying psychopathology
Munchausen's syndrome	Factitious disorder ("hospital addiction") — repeated feigned acute illness to assume the sick role; no external gain
Munchausen's by proxy (Polle syndrome)	Fabricating/inducing illness in another (usually a child); controversial diagnosis

SUPPLEMENTARY — "CULTURE-BOUND" SYNDROMES (NOW SEEN CROSS-CULTURALLY) **Koro** (penis-shrinking, fatal; anxiety-related) · **Amok** (dissociative/ depressive, SE Asia) · **Dhat** (psychosexual, Asia) · **Windigo** (cannibalistic delusion, Native American) · **Susto** (soul-loss anxiety, Central/South America) · **Latah** (startle automatic obedience, echopraxia; SE Asia) · **Piblokto** (screaming/running naked in snow; Arctic).

VIVA TRAP — CAPGRAS VS FREGOLI VS INTERMETAMORPHOSIS **Capgras:** familiar person replaced by a *double* (same look, "wrong" person). **Fregoli:** strangers are really *one* familiar persecutor in *disguise*. **Intermetamorphosis:** people have swapped identities *and* appearance. All are delusional misidentification syndromes.

APPENDIX II

Defence Mechanisms & Cognitive Distortions

Fish's Clinical Psychopathology, 5th ed., Appendix II (pp. 127–131)

Defence mechanisms

DEFINITION Techniques the psyche uses to protect itself from overwhelming anxiety or stress — explanatory concepts derived from psychoanalysis, not entities in themselves.

Defence	Meaning / example
Altruism	Meeting one's own needs through others' lives (pushing a child into the career one wanted)
Denial	Refusal to acknowledge a threatening reality ("it can't happen here"); persists despite facts
Displacement	Shifting emotion from one object to a less threatening one (anger at boss shown as irritability at home)
Idealisation	Ascribing omnipotence to another ("you will save me")
Identification with the aggressor	Victim adopts the opponent's qualities (battered spouse justifying the abuser; Stockholm syndrome)
Projection	Attributing one's own unacceptable impulses/ attributes to an external source
Projective identification	Projecting an aspect of self onto another and coercing them to behave accordingly (key in borderline PD)

Defence	Meaning / example
Rationalisation	Finding excuses to justify unacceptable behaviour ("it was OK because he hit me first")
Reaction formation	Denying an unacceptable impulse and adopting the opposite (morality crusades)
Repression	Unconscious forgetting of painful ideas/ impulses; overlaps with denial
Splitting	Dividing people into all-good or all-bad (characteristic of borderline PD)
Sublimation	Transferring unacceptable urges into acceptable channels (anger political activism)
Somatosensory amplification	Experiencing bodily sensations as unusually intense/distressing (underpins somatisation)

Cognitive distortions (after Burns; Ellis & Grieger)

DEFINITION Errors in thinking that distort the person's view of self, others and the future.

- **All-or-nothing thinking** — black-and-white; anything short of perfect = total failure.
- **Approval-seeking** — "I must be loved/approved of always."
- **Comparison** — constantly measuring oneself against others on scanty evidence.
- **Disqualifying the positive** — rejecting good experiences as "not counting".
- **Emotional reasoning** — "I feel it, therefore it's true."
- **Fallacy of fairness** — judging a non-justice event as "unfair".
- **Jumping to conclusions** — *mind reading* (assuming others' negative thoughts) & *fortune-telling* (predicting bad outcomes).
- **Labelling / mislabelling** — attaching a global negative label to self/others ("I'm a slob").
- **Magnification (catastrophising) / minimisation** — the "binocular trick".

MNEMONIC — BORDERLINE-LINKED DEFENCES Borderline PD "**Split & Project**": **Splitting** + **Projective identification** are the hallmark immature defences.

L2 **Distinguish projection from projective identification.**

Projection simply attributes one's own unacceptable impulse/attribute to someone else. Projective identification goes further: the projected aspect is placed into another person who is then unconsciously coerced into actually *behaving* in line with it, creating a shared enactment — especially relevant to therapists working with borderline patients.

L2 **What is splitting and why does it disrupt ward teams?**

Splitting is the inability to integrate good and bad, so people are seen as all-good or all-bad. In borderline PD it causes shifting idealisation/devaluation of staff, setting team members against one another (e.g., nurses vs doctors) — hence the need for team awareness and consistency.

EXAM TOOLKIT

High-Yield Comparison Tables

Each pair gives: definitions · key distinguishing feature · a clinical example (fictional) · the common viva trap · a one-line memory hook.

Illusion vs Hallucination

	Illusion	Hallucination
Definition	Misinterpretation of a *real* external stimulus	Perception *without* an adequate external object
Key difference	Object present, read wrongly	No object; arises within, felt as from without
Example	Shadow seen as an attacker on a dark road	Hearing a clear voice in an empty, silent room
Viva trap	Illusions are *not* pathological in themselves	Modality steers differential (visual organic; auditory in clear consciousness schizophrenia)
Memory hook	ILL-reading of something real	No object at a-LL

Hallucination vs Mental Image / Pseudo-hallucination

	True hallucination	Mental image	Pseudo-hallucination
Space	Objective (outer)	Subjective (inner)	Subjective (inner, "in the head")
Substantiality	Full, fresh, substantial	Sketchy	Vivid but non-substantial

	True hallucination	Mental image	Pseudo-hallucination
Will	Involuntary	Voluntary (can re-create)	Involuntary
Insight	Taken as real	Known to be internal	Known not to be a real perception
Viva trap	Jaspers defines pseudo-hallucination by *space*; Hare by *insight* — Fish favours a continuum, SCAN avoids the term.		

Obsession vs Delusion vs Overvalued Idea

	Obsession	Delusion	Overvalued idea
Ownership	Recognised as own	Own belief, full conviction	Own belief
Resistance / insight	Resisted; insight retained (ego-dystonic)	Not resisted; insight absent	Not resisted; some insight
Fixity / plausibility	Intrusive, repugnant	Unshakeable, implausible, culturally incongruent	Strong feeling tone, long-lasting, plausible, some basis in reality
Example	"I must check the gas 20× though I know it's off"	"MI5 implanted a chip in my tooth"	A fitness fanatic convinced their diet is uniquely correct
Viva trap	The patient's *attitude* (resistance, conviction, ownership) separates them — not the content.		

Circumstantiality vs Tangentiality

	Circumstantiality	Tangentiality
Definition	Excessive trivial detail but the goal is *eventually reached*	Drifts away from the point and *never reaches* it
Goal	Reached (by a winding path)	Lost
Example	Asked about sleep, gives a 5-minute history of the bedroom, then answers	Asked about sleep, ends up discussing the neighbour's dog, never answering
Memory hook	Circles back	Trails off on a tangent

> **NOTE** Fish's text details *circumstantiality* explicitly; "tangentiality" is standard descriptive-psychopathology/DSM terminology given here for contrast — closely related to Fish's "derailment/loosening".

Flight of Ideas vs Loosening of Associations (Derailment)

	Flight of ideas	Loosening of associations / derailment
Links between thoughts	**Understandable** (chance, clang, rhyme, puns)	**Not understandable** (slides to unrelated subsidiary ideas)
Speed	Fast, pressured	Not necessarily fast
Typical of	Mania	Schizophrenia (formal thought disorder)
Viva trap	The test is whether *you can follow the connections*. Followable flight of ideas. Not followable loosening.	

Thought Blocking vs Poverty of Thought

	Thought blocking	Poverty of thought
Definition	Sudden *arrest* of the thought stream leaving a blank; a new thought may follow	Global reduction in the *amount* of thought/ideas
Onset	Abrupt, mid-stream	Pervasive, sustained
Patient experience	May be terrifying if insight retained	Experienced as emptiness/slowness
Association	Clear blocking suggestive of schizophrenia	Negative symptoms; depression (as slowing)

Thought Insertion vs Thought Withdrawal vs Broadcasting

Insertion	Withdrawal (deprivation)	Broadcasting (diffusion)
Foreign thoughts *put into* the mind from outside	Own thoughts *taken out* by an external agency	Thoughts are shared/ known to others as one thinks
All are forms of *thought alienation* (now ICD-11 schizophrenia criteria). Record the patient's wording verbatim, as broadcasting has several definitions.		

Depersonalisation vs Derealisation

	Depersonalisation	Derealisation
Focus	The **self** feels unreal/ changed/detached	The **environment** feels unreal/flat/distant

	Depersonalisation	Derealisation
Quality	Unpleasant, insight retained, "as if"	Unpleasant, insight retained, "behind glass"
Viva trap	Neither is a delusion — distinguish from nihilistic (Cotard) delusion, which is held with conviction.	
Memory hook	"*I* am unreal"	"The *world* is unreal"

Catatonic vs Depressive Stupor

Feature	Catatonic	Depressive
Face	Deadpan (lively eyes)	Depressed/anxious
Tone / obstruction	Increased / variable + obstruction	Normal
Catalepsy, stereotypy, waxy flexibility	Present	Absent
Emotional reactivity	Absent	Present (worse on affect-laden topics)
Incontinence	Common	Absent

Confabulation vs Pseudologia Fantastica vs Delusional Memory

	Confabulation	Pseudologia fantastica	Delusional memory
Basis	Organic; clear consciousness	Non-organic (personality)	Psychotic
Nature			

	Confabulation	Pseudologia fantastica	Delusional memory
	Fills memory gaps unintentionally	Fluent plausible lying, often at crisis	A memory with delusional significance / a "remembered" event that never occurred
On confrontation	Not corrected	Admits lying with proof	Held with conviction

EXAM TOOLKIT

Master Glossary of Terms

Alphabetical. Each entry: precise meaning · plain-language gloss where useful · the chapter it belongs to. Terms are drawn from Fish's 5th edition.

Term	Meaning (plain gloss in brackets)	Ch
Affect	Short-lived present emotional responsiveness, inferred from face/ body (the "weather"); may be congruent/incongruent with mood	6
Affective incontinence	Total loss of control of emotional expression (forced laughing/weeping) with mismatch to feeling	6
Akathisia	Motor restlessness/inability to keep still with subjective tension (antipsychotic effect)	8
Ambitendency	Tentative to-and-fro movements never reaching the goal (ambivalence in action)	8
Amnesia (antero-/ retrograde)	Loss of memory for events after / before an insult	5
Anhedonia	(Loss of the capacity for interest or pleasure) — core to depression (Ribot, 1896)	6
Anosognosia	(Denial of one's own illness/deficit), e.g. denying hemiplegia after stroke	3
Apophany	The primary delusional experience (new meaning arising de novo)	4
		4

Term	Meaning (plain gloss in brackets)	Ch
Autochthonous delusion	Sudden delusional idea appearing fully formed "out of the blue"	
Autoscopy	Seeing oneself and knowing it is oneself (phantom mirror-image)	3
Blunting of affect (parathymia)	Loss of emotional finesse/social sensitivity	6
Catalepsy	Maintaining imposed/fortuitous postures for long periods (without waxy resistance)	8
Circumstantiality	(Many trivial details but the point is eventually reached)	4
Clang association	Thoughts linked by sound/rhyme rather than meaning (in flight of ideas)	4
Confabulation	Organic filling of memory gaps with untrue material in clear consciousness	5
Déjà vu / jamais vu	Feeling a new event is familiar / a familiar event feels unfamiliar	5
Delusion	False, unshakeable belief out of keeping with culture (primary vs secondary)	4
Delusional mood (Wahnstimmung)	Pervasive sense that "something concerning me is going on" (meaning not yet known)	4
Delusional perception	New self-referential meaning attached to a normally perceived object ("two-memberedness")	4
Demoralisation		6

Term	Meaning (plain gloss in brackets)	Ch
	Despair/feeling trapped with external source; anhedonia & guilt absent, motivation intact	
Depersonalisation	(Feeling unreal/detached from oneself), insight retained	7
Derealisation	(Feeling the environment is unreal/ distant)	7
Dissociation of affect	Absence of emotional response where some is expected (defence)	6
Drivelling thinking	Constituents of a complex thought muddled together; organisation lost	4
Dysmegalopsia	(Change in perceived size/shape) — micropsia, macropsia, metamorphopsia	3
Echolalia / echopraxia	Echoing others' speech / imitating their actions	8
Ecstasy	Exalted rapturous state (distinct from elation)	6
Ego-syntonic / -dystonic	(Compatible with / distressingly incompatible with one's self-concept)	1,4
Extracampine hallucination	Hallucination outside the sensory field (e.g., "behind me" while facing forward)	3
Flight of ideas	Rapid, no-direction thought with understandable chance links (mania)	4
Formal thought disorder	Disorder of the *form*/connection of thinking (schizophrenia, organic)	4
Formication	(Feeling of small animals crawling on the skin); "cocaine bug"	3

Term	Meaning (plain gloss in brackets)	Ch
Functional hallucination	Stimulus + hallucination perceived together, same modality	3
Gedankenlautwerden	Hearing one's own thoughts aloud as/ just before they occur	3,4
Gegenhalten	Opposing passive movement with equal force	8
Hyperaesthesia / hyperacusis	(Heightened sensation / sound sensitivity)	3
Hypnagogic / hypnopompic	Hallucinations when falling asleep / waking (not pathological)	3
Illusion	Misinterpretation of a real external stimulus (completion/affect/pareidolia)	3
Incongruity of affect	Emotion in the wrong direction for the context	6
Katathymic amnesia	Motivated forgetting of specific painful memories (no identity loss)	5
Lability of affect	Rapid abrupt mood changes unrelated to stimuli	6
Lilliputian hallucination	Hallucination of tiny people/objects, usually with pleasure	3
Logoclonia / palilalia	Repeating last syllable / repeating a word with rising frequency	8
Mannerism	Odd, stilted but still goal-directed movement/posture	8
Mitgehen / Mitmachen	Moving with the slightest pressure / allowing passive movement despite instruction to resist	8

Term	Meaning (plain gloss in brackets)	Ch
Mood	Pervasive, sustained emotion colouring one's outlook (the "climate")	6
Moria / Witzelsucht	Frontal euphoria with facetious humour/silliness, lack of foresight	1,6
Mutism	Complete loss of speech	4,8
Negativism	Motiveless resistance (passive) or doing the opposite (active/command)	8
Neologism	New word, or ordinary word used in a new way (schizophrenia)	4
Nihilistic delusion	Delusion of non-existence of self/body/ world (Cotard)	4
Obsession (rumination)	Persistent intrusive thought, recognised as one's own, resisted	4
Overvalued idea	Dominant, long-lasting, emotionally charged, plausible idea	4
Pareidolia	Vivid effortless illusion (faces in clouds/ fire)	3
Passivity phenomena	Actions/impulses/feelings experienced as "made" by an external force	7,8
Perseveration	(Inappropriate continuation/repetition beyond relevance) — organic	4,8
Phantom limb	Commonest organic somatic hallucination after amputation	3
Prolixity	"Ordered flight of ideas" (hypomania) — returns to the goal	4
Pseudo-hallucination		3

Term	Meaning (plain gloss in brackets)	Ch
	Vivid non-substantial image in inner space, known not real, involuntary	
Pseudologia fantastica	Fluent plausible/pathological lying without organic cause	5
Psychomotor retardation	Generalised slowing of thought/ movement (depression) — "steady brake"	6,8
Reflex hallucination	Stimulus in one modality hallucination in another (morbid synaesthesia)	3
Ribot's law	Recent memories lost before remote ones	5
Schizophasia	"Word salad" / speech confusion (a thought disorder)	4
Schnauzkrampf	Snout spasm (lips thrust forward) in catatonia	8
Sperrung	Blocking/obstruction of motor acts (catatonia) — "rod in the spokes"	8
Stereotypy	Repetitive, uniform, non-goal-directed movement/utterance	8
Stupor	Near-complete loss of activity, no response to stimuli	8
Synaesthesia	A stimulus in one sense producing experience in another	3
Tardive dyskinesia	Repetitive purposeless orofacial movements (often antipsychotic-related)	8
Thought alienation		4,7

Term	Meaning (plain gloss in brackets)	Ch
	Thoughts under external control / shared (insertion, withdrawal, broadcasting)	
Thought blocking	Sudden arrest of the thought stream	4
Twilight state	Restricted consciousness, broken continuity, ordered behaviour (often epileptic)	9
Vorbeireden	Talking past the point / approximate answers (Ganser)	4,5
Waxy flexibility (flexibilitas cerea)	Plastic "soft-wax" resistance; imposed posture then held	8
Witzelsucht	Facetious, inappropriate joking in frontal-lobe damage	1

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Mental Status Examination — Application & Documentation

Where each Fish phenomenon sits in the MSE, how to identify it, and how to document it objectively. **Describe the form, quote the patient, separate report from interpretation.**

Fish phenomena mapped to MSE domains

MSE domain	Phenomena (Fish chapter)	How elicited
Appearance & behaviour	Motor signs — retardation, mannerisms, stereotypies, catatonia, negativism, waxy flexibility, tics, drug-induced movements (Ch 8); expressive movements (Ch 6, 8)	Observation + gentle passive-movement testing (Mitgehen, catalepsy, Gegenhalten)
Speech	Rate/flow (pressure, poverty), stammer, mutism, neologisms, schizophasia, verbigeration, Vorbeireden (Ch 4, 8)	Listen to form & flow; note vs record content separately
Mood & affect	Mood (subjective), affect (observed), range/congruity, lability, blunting, anhedonia, demoralisation (Ch 6)	"How have your spirits been?" + observe reactivity and the smile-of-the-eyes test
Thought — stream	Flight of ideas, circumstantiality, perseveration, blocking (Ch 4)	Follow whether links are understandable and whether goal is reached
Thought — possession	Obsessions/compulsions, thought insertion/	"Are your thoughts your own? Could anyone interfere with them?"

MSE domain	Phenomena (Fish chapter)	How elicited
	withdrawal/broadcasting (Ch 4, 7)	
Thought — content	Delusions (all types), overvalued ideas (Ch 4)	Explore conviction, fixity, cultural context, primary vs secondary
Perception	Illusions, hallucinations (all modalities), functional/reflex/ extracampine, sensory distortions (Ch 3)	Fish's probe questions (Ch 3 box); clarify location, insight, modality, relation to stimuli
Cognition	Orientation, attention, memory, amnesias, paramnesias, consciousness level (Ch 5, 9)	Orientation (time place person), attention/registration tasks, recall; note confabulation
Experience of self	Depersonalisation, derealisation, passivity, loss of self-continuity (Ch 7)	Fish's self-experience probes (Ch 7 box)
Insight & judgement	Degree of insight (pervasive — pseudo-hallucination debate), attitude to symptoms (Ch 3, 1)	Ask the patient's own explanation of the experiences

COMMON DOCUMENTATION ERRORS

- Writing a **diagnosis instead of a sign** ("psychotic") rather than describing the phenomenon.
- Recording **content as if it were form** ("heard a voice saying X" without stating modality, 2nd/3rd person, clarity, insight).
- Calling "people talk about me" an **auditory hallucination** when it may be a delusion with no perception.
- Labelling resistance "negativism" when it is actually goal-directed refusal.
- Not quoting the patient **verbatim** for thought broadcasting (which has several definitions).

MODEL DOCUMENTATION (FICTIONAL)

Perception: "Reports hearing two male voices conversing *about* him in the third person, clearly audible, located in external space, present daily, in clear consciousness; attributes them to neighbours; no insight that they are illness-related."
Thought possession: "States 'thoughts are pulled out of my head by a machine' — describes thought withdrawal; records belief verbatim."
Note: describes the form, quotes the patient, and separates the patient's attribution (interpretation) from the observed phenomenon.

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Mnemonics Collection

PERCEPTION

- **Illusion vs hallucination:** ILL-reading of something real vs no object at a-LL.
- **Three illusions — "CAP":** Completion, Affect, Pareidolia.
- **Functional vs reflex hallucination:** Functional = same modality (stimulus still perceived); Reflex = different modality.
- **Size distortions:** miCROpsia = CRushed/small; maCROpsia = CRoissant/large.

THOUGHT & SPEECH

- **Schneider's 5 FTD features — "DSOFD":** Derailment, Substitution, Omission, Fusion, Drivelling.
- **Primary delusional experiences:** Mood Perception Idea (Wahnstimmung delusional perception autochthonous idea).
- **Circumstantial vs tangential:** Circumstantial Comes back; Tangential Trails off.
- **Flight vs loosening:** If you can follow it = Flight; if you can't = Loosening.

EMOTION & SELF

- **Five levels of emotion:** Normal Always Avoids Morbid Madness (reactions reactions expressions expressions disorders).
- **Jaspers' 4 aspects of self — "AUCB":** Activity, Unity, Continuity, Boundaries.
- **Depersonalisation vs derealisation:** "I" vs "the world".

MOTOR & CONSCIOUSNESS

- **Compliance spectrum:** Gegenhalten fights Mitmachen goes along Mitgehen goes all the way.
- **Retardation vs obstruction:** steady Brake vs Rod in the spokes.
- **3 changes of consciousness — "DLR":** Dream-like, Lowered, Restricted.
- **Orientation lost — "TPP":** Time, Place, Person.

PERSONALITY

- **Clusters:** A = Weird (odd), B = Wild (dramatic), C = Worried (anxious).
- **ICD-11 domains:** Negative affectivity, Detachment, Dissociality, Disinhibition, Anankastia (+ Borderline qualifier).

- **Borderline defences:** "Split & Project" (splitting + projective identification).

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Long-Answer Questions — Structured Frameworks

Skeletons for essay/long-viva answers. Expand each heading with the chapter detail above.

Q1 · "Discuss the classification and clinical significance of hallucinations."

Framework: (1) Define (Jaspers; SCAN). (2) Distinguish from illusion, mental image, pseudo-hallucination. (3) Classify by *modality* (auditory/visual/olfactory/gustatory/tactile/visceral) with diagnostic pointers. (4) Special types (functional, reflex, extracampine, autoscopy, hypnagogic/hypnopompic). (5) Hallucinatory syndromes (alcoholic, organic). (6) Causes (emotion, suggestion, sense-organ disease, deprivation, CNS). (7) Clinical significance: modality steers differential; visual organic; continuous auditory in clear consciousness schizophrenia; risk from imperative/persecutory voices. (8) Non-clinical hallucinatory-like experiences. (9) Conclude on careful phenomenological assessment over labels.

Q2 · "Compare and contrast delusions, overvalued ideas and obsessions."

Framework: Define each. Tabulate ownership, insight, resistance, fixity, plausibility, cultural congruence. Primary vs secondary delusions (Schneider's three primary experiences). Overvalued idea's feeling tone and reality basis. Obsession's ego-dystonic, resisted quality. Differential pitfalls (OCD with poor insight vs delusion; morbid jealousy as delusion vs overvalued idea). Clinical/forensic significance (risk). Conclude: the *attitude of the patient*, not content, is decisive.

Q3 · "Describe the motor disorders seen in catatonia."

Framework: Subjective (passivity) vs objective. Adaptive (obstruction/Sperrung, mannerisms). Non-adaptive (stereotypy, perseveration, verbigeration). Induced/compliance spectrum (automatic obedience, echolalia/echopraxia, Mitmachen, Mitgehen, Gegenhalten, negativism, ambitendency, forced grasping). Posture (waxy flexibility, catalepsy, psychological pillow, Schnauzkrampf). Complex behaviour (stupor vs excitement). Ungvari's four syndromal classes. Conclude with examination technique and need to exclude organic causes.

Q4 · "Outline the disorders of memory and their clinical and forensic relevance."

Framework: Memory types/processes. Amnesias (organic: retrograde/anterograde, amnestic state/Korsakoff, Ribot's law; psychogenic: dissociative, katathymic). Paramnesias of recall (retrospective falsification, false memory, screen memory, confabulation, pseudologia, cryptamnesia, delusional memory) and recognition (déjà/jamais vu, false recognition, Capgras). Hyperamnesia/flashbacks. Forensic relevance (false memory, false confession, dissociative amnesia vs malingering, memory's reconstructive nature).

Q5 · "Describe the levels of disordered emotion in Fish, with examples."

Framework: Vocabulary (feeling/emotion/mood/affect). Five levels: normal reactions abnormal reactions (adjustment; anxiety, phobia, demoralisation; Yerkes–Dodson) abnormal expressions (dissociation of affect, belle indifférence, denial, apathy, perplexity) morbid expressions (blunting, incongruity, flattening, lability, affective incontinence, smiling depressive) morbid disorders (depressive illness with anhedonia/retardation/loss of resonance; morbid anxiety; euphoria/ecstasy; moria). Clinical significance (suicide risk, differentiating normal from pathological).

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Clinical Case-Based Viva (Fictional Illustrations)

All vignettes are invented for teaching. Do not diagnose from a single symptom.

Case 1 · The "voice from the tap"

Vignette: A 28-year-old says that *only* when the bathroom tap runs does he also hear a voice naming him; he hears the water *and* the voice as separate sounds; when the tap is off, the voice stops.
Phenomenon: Functional hallucination.
Supporting features: stimulus and hallucination perceived together, same modality, stimulus not transformed, hallucination stimulus-dependent.
Rule out: illusion (stimulus would be *transformed*); reflex hallucination (different modality).
Clarifying question: "When the voice comes, can you still hear the water at the same time, or does the water *become* the voice?"
Model answer: "This is a functional hallucination, not an illusion, because the real stimulus (running water) is perceived simultaneously with and separately from the hallucination, which is abolished when the stimulus stops."

Case 2 · Multiple phenomena together

Vignette: A 35-year-old describes: (a) thoughts suddenly vanishing mid-sentence "as if someone deletes them"; (b) a certainty that a stranger on TV is in love with her; (c) she dates this belief to "always", though her family say it began 3 weeks ago.
Phenomena: (a) thought withdrawal (thought alienation) — possibly experienced as thought blocking; (b) erotomania (de Clérambault); (c) retrospective delusion (backdating).
Distinguish: (a) thought withdrawal (foreign agency) vs ordinary losing one's thread (fatigue); (b) erotomania vs an overvalued romantic preoccupation (test conviction/fixity); (c) genuine chronicity vs retrospective delusional dating (collateral history).
Clarifying questions: "When the thought disappears, do you feel something/someone *takes* it?"; "How do you know this person loves you?"; corroborate timeline with family.

Case 3 · Stupor on the ward

Vignette: A mute, immobile patient; the examiner raises the arm to take the pulse and it remains suspended after release; the face is deadpan but the eyes track the examiner; incontinent of urine.
Phenomenon: Catatonic stupor with catalepsy (and likely increased tone/obstruction).
Distinguish from depressive stupor: depressive stupor shows a depressed facies, normal tone, no catalepsy/stereotypy, preserved emotional reactivity, no incontinence.
Clarifying/examination steps: test for waxy flexibility, Mitgehen, Gegenhalten; probe emotional reactivity on affect-laden topics; **always exclude organic causes** (bloods, imaging, EEG, LP) in rapidly developing stupor.

Case 4 · Confabulation vs lying

Vignette: A 60-year-old with a long alcohol history fills gaps in his recent memory with detailed but untrue "events", maintained in clear consciousness, not corrected when challenged; he is apathetic with poor recent recall.
Phenomenon: Confabulation (suggestive of Korsakoff's amnestic syndrome).
Distinguish: pseudologia fantastica (non-organic, admits lying on proof); ordinary lying (conscious, for gain); delusional memory (held with psychotic conviction).
Clarifying steps: formal memory testing, collateral history, assess for thiamine deficiency; note the clear consciousness + organic context pointing to confabulation.

Case 5 · "The world feels unreal"

Vignette: A 22-year-old with panic attacks says the world looks "flat, like behind glass" and she feels "like a robot watching herself"; she knows this is not actually true and finds it distressing.
Phenomena: Derealisation (world) + depersonalisation (self); insight retained.
Distinguish: nihilistic (Cotard) delusion — would be held with conviction ("I *am* dead"); schizoid detachment — lifelong, non-distressing.
Clarifying question: "Do you truly believe you are unreal, or does it just *feel* that way while you know it isn't?" (insight retained depersonalisation, not delusion).

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Rapid-Revision Sheets by Domain

PERCEPTION (CH 3) Distortions (intensity/quality/space/time) · Deceptions = illusions (completion/affect/pareidolia) + hallucinations (perception without object; Jaspers). Modalities: auditory (voices, Gedankenlautwerden), visual (organic; Lilliputian), olfactory, gustatory, tactile (formication), visceral. Special: functional, reflex, extracampine, autoscopy, hypnagogic/hypnopompic. Syndromes: alcoholic & organic hallucinosis. Body image: hyper-/hypo-/para-schemazia, anosognosia.

THOUGHT & SPEECH (CH 4) Stream: flight of ideas, prolixity, slowing, circumstantiality, perseveration, blocking. Possession: obsessions/compulsions, thought insertion/withdrawal/broadcasting. Content: delusions (primary vs secondary; persecution, infidelity, love, grandiose, ill-health, guilt, nihilistic, poverty), overvalued ideas. Form: FTD (Bleuler/Cameron/Goldstein/Schneider; transitory/drivelling/desultory). Speech: mutism, Vorbeireden, neologism, schizophasia.

MEMORY (CH 5) Types (sensory/STM/LTM; explicit vs implicit). Amnesias: retrograde/anterograde, amnestic state (Korsakoff), dissociative, katathymic; Ribot's law. Paramnesias of recall (retrospective falsification, false memory, screen memory, confabulation, pseudologia, cryptamnesia, delusional memory) & recognition (déjà/jamais vu, Capgras). Hyperamnesia/flashbacks/eidetic.

EMOTION (CH 6) Feeling/emotion/mood/affect. Five levels: normal abnormal reactions (anxiety, phobia, demoralisation) abnormal expressions (dissociation of affect, belle indifférence, denial, apathy, perplexity) morbid expressions (blunting, incongruity, flattening, lability, affective incontinence, smiling depressive) morbid disorders (depression + anhedonia; euphoria/ecstasy; moria).

SELF (CH 7) Jaspers' 4 aspects (Activity, Unity, Continuity, Boundaries). Depersonalisation/derealisation (insight retained, unpleasant). Loss of emotional resonance. DID. Passivity/"made" experiences. Theory of mind; self-disorder model (Sass & Parnas).

MOTOR (CH 8) Subjective passivity. Adaptive (retardation vs obstruction; mannerism). Non-adaptive (tic, stereotypy, perseveration, verbigeration). Induced spectrum (automatic obedience, echolalia/echopraxia, Mitmachen, Mitgehen, Gegenhalten, negativism, ambitendency). Posture (waxy flexibility, catalepsy). Stupor vs excitement. Drug-induced: akathisia, dystonia, tardive dyskinesia.

CONSCIOUSNESS (CH 9) Attention (active/passive, distractibility). Orientation Time Place Person. Three changes: Dream-like (delirium — visual hallucinations, fear), Lowered (torpor), Restricted (twilight state — often epileptic). Fugue (not always hysterical).

PERSONALITY (CH 10) Define (Schneider). Assess after recovery, use informants, require negative impact. DSM-5 clusters A/B/C (10 types). ICD-11 dimensional: severity + 5 trait domains + borderline qualifier. Key differentials: schizoid vs avoidant vs depersonalisation; antisocial vs borderline; anankastic over-diagnosis.

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Commonly Confused Terms — Quick Separators

Pair	One-line separator
Illusion / Hallucination	Real object misread / no object at all
Hallucination / Pseudo-hallucination	Outer space, taken as real / inner space, known not real
Functional / Reflex hallucination	Same modality, stimulus co-perceived / different modality
Illusion / Functional hallucination	Stimulus transformed / stimulus + hallucination separate
Obsession / Delusion	Resisted, own, insight / unshakeable, no insight
Obsession / Thought insertion	My thought, against my will / not my thought, put in
Delusion / Overvalued idea	Implausible, fixed, indifferent to others / plausible, feeling-toned, some reality basis
Delusional perception / misinterpretation	New meaning NOT derivable from mood / meaning derived from existing belief
Circumstantiality / Tangentiality	Reaches the point / never reaches it
Flight of ideas / Loosening	Understandable links / unintelligible links
Thought blocking / Poverty of thought	Sudden arrest / global reduction in amount
	Put in from outside / taken out by outside

Pair	One-line separator
Thought insertion / withdrawal	
Depersonalisation / Derealisation	Self unreal / world unreal
Depersonalisation / Nihilistic delusion	"As if" with insight / held with conviction
Mannerism / Stereotypy	Odd goal-directed act / repetitive non-goal-directed act
Psychomotor retardation / Obstruction	Steady uniform slowing / irregular sudden block
Waxy flexibility / Catalepsy	Plastic resistance felt / posture held without that resistance
Mitmachen / Mitgehen	Moderate effort to move / slightest touch moves them
Confabulation / Pseudologia fantastica	Organic, unintentional / non-organic, admitted on proof
Déjà vu / Jamais vu	New feels familiar / familiar feels new
Capgras / Fregoli	Familiar person is a double / strangers are one persecutor in disguise
Blunting / Incongruity of affect	Reduced finesse (right direction) / wrong direction
Schizoid / Avoidant PD	No desire for relationships / strong desire but held back
Hypnagogic / Hypnopompic	Falling asleep / waking up
Delirium / Dementia	Acute, clouded, fluctuating, visual hallucinations / chronic, clear consciousness

EXAM TOOLKIT

Chapter & Concept Completion Checklist

Every accessible chapter and appendix of the 5th edition has been reviewed and covered.

Chapter	Pages	Status	Key concepts covered
1 · Classification	1–10		Syndrome/disease, primary/secondary, organic/functional, neurosis/psychosis, DSM-5 vs ICD-11
2 · What is Psychopathology?	11–23		Jaspers, form/content, descriptive/explanatory, realism/constructivism/ pragmatism, validity/ reliability, clinical significance
3 · Perception	24–41		Distortions, illusions, hallucinations (all modalities + special types), hallucinosis, body image
4 · Thought & Speech	42–62		Stream, possession, content (delusions), form (FTD), speech disorders
5 · Memory	63–70		Amnesias, paramnesias (recall & recognition), hyperamnesia
6 · Emotion	71–79		Five levels, anxiety/ phobia/demoralisation, blunting/incongruity/

Chapter	Pages	Status	Key concepts covered
			lability, morbid depression/mania/ ecstasy
7 · Experience of Self	80–85		Jaspers' 4 aspects, depersonalisation/ derealisation, passivity, DID, theory of mind
8 · Motor Disorders	86–102		Passivity, adaptive/non-adaptive, induced/ compliance, posture, stupor/excitement, drug-induced
9 · Consciousness	103–107		Attention, orientation, dream-like/lowered/ restricted, delirium, twilight, fugue
10 · Personality Disorders	108–121		Definition, assessment, DSM-5 clusters & categories, ICD-11 dimensional model
Appendix I · Psychiatric Syndromes	122–126		~20 named syndromes + culture-bound disorders
Appendix II · Defences & Distortions	127–131		Defence mechanisms + cognitive distortions
Index	132+	(navigational)	Used to confirm coverage; not reproduced

EXAM TOOLKIT

References, Accuracy Statement & Final Audit

Principal source (verified from the PDF)

FULL BIBLIOGRAPHIC CITATION Casey, P. & Kelly, B. (2024). *Fish's Clinical Psychopathology: Signs and Symptoms in Psychiatry* (5th ed.). Cambridge: Cambridge University Press. © The Royal College of Psychiatrists 2024. ISBN 978-1-009-37269-5 (paperback); e-ISBN 9781009372688. DOI 10.1017/9781009372688. Original author: Frank Fish (1st ed., 1967). 152 PDF pages; 10 chapters + 2 appendices + index.

Edition lineage (from the PDF): 1st 1967 (Fish) revised 1974 & 2nd 1985 (ed. Hamilton) 3rd 2007 4th 2019 5th 2024 (Casey & Kelly). Chapter 2 ("controversies") was added in the 4th edition. ICD-11 referencing added in the 5th.

Accuracy & copyright statement

- All definitions, page references and the chapter sequence are taken directly from the supplied 5th-edition PDF.
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Final audit

CHAPTERS REVIEWED All 10 chapters, both appendices, preface and front matter of the 5th-edition PDF were read in full and are covered.

CLARIFICATION ON THE SECOND "FISH" PDF The task referred to a possible *"Fish Clinical Cytology"* file. The repository was searched thoroughly: **no such file exists**, and no book by that title exists in Fish's catalogue. The only Fish title present is *Fish's Clinical Psychopathology* (the file analysed here). Per your instruction ("yes use that only — Fish file is

the Clinical Psychopathology only"), these notes are built solely from the Clinical Psychopathology 5th edition. No cytology content was fabricated.

UNRESOLVED LIMITATIONS (1) The PDF's embedded text has minor OCR artefacts (e.g., stray characters in the page footers), which were ignored. (2) A handful of secondary citations inside Fish (e.g., Sims, Oyebode, Jaspers, Hamilton) are named as Fish cites them but were not independently consulted. (3) Page numbers refer to the book's printed pagination as shown in the PDF. (4) Where modern DSM-5-TR terminology differs from Fish's descriptive terms, both are given without overwriting Fish.

Supplementary sources consulted for cross-terminology

Only standard, widely-accepted descriptive-psychopathology and nosology references were used for clearly-flagged supplementary notes: the DSM-5/DSM-5-TR (American Psychiatric Association) and ICD-11 (WHO) classifications as already cited within Fish, and the standard descriptive terms (e.g., tangentiality) as used in Sims' *Symptoms in the Mind*, which Fish itself references.

Fish's Clinical Psychopathology — 5th Edition: Comprehensive Study & Viva Notes

Based on Casey & Kelly (2024), Cambridge University Press · © The Royal College of Psychiatrists 2024 (source text). These notes are an independent educational study aid; all clinical examples are fictional.

Made by Tejas Dangodra | M.Phil. Clinical Psychology